

#1 · Assessment and Diagnosis

What does the ICF framework stand for?

Answer

ICF stands for the International Classification of Functioning, Disability and Health. It is the World Health Organization model that Canadian physiotherapists use to describe a client's health condition in terms of body structure and function, activity, and participation, all influenced by environmental and personal factors.

#2 · Assessment and Diagnosis

Define impairment in the ICF model.

Answer

Impairment refers to a problem in body function or structure, such as reduced strength, restricted joint range, or abnormal sensation. It is one of three components of functioning alongside activity limitation and participation restriction.

#3 · Assessment and Diagnosis

Define activity limitation in the ICF model.

Answer

Activity limitation is a difficulty an individual has in executing a task or action, such as walking, climbing stairs, or dressing. It links body-level impairments to real-world functional performance.

#4 · Assessment and Diagnosis

Define participation restriction in the ICF model.

Answer

Participation restriction is a problem an individual experiences in involvement in life situations, such as returning to work, sport, or social roles. It captures the broader social impact of a health condition beyond simple task performance.

#5 · Assessment and Diagnosis

How do contextual factors fit into the ICF model?

Answer

Contextual factors are split into environmental factors, like physical surroundings, social support, and attitudes, and personal factors, like age, coping style, and motivation. Both can act as facilitators or barriers to a client's functioning and are considered during assessment and goal setting.

#6 · Assessment and Diagnosis

What is a physiotherapy diagnosis?

Answer

A physiotherapy diagnosis is the clinical judgment describing the movement dysfunction, its underlying impairments, and the impact on activity and participation. It differs from a medical diagnosis because it focuses on movement and function rather than pathology alone, and it directly guides treatment planning.

#7 · Assessment and Diagnosis

What is clinical reasoning in physiotherapy assessment?

Answer

Clinical reasoning is the thinking process used to gather, interpret, and analyze information to form a diagnosis and plan. It blends hypothetico-deductive reasoning, where the therapist tests hypotheses, with pattern recognition built from clinical experience.

#8 · Assessment and Diagnosis

What is the purpose of an irritability assessment?

Answer

Irritability describes how easily a tissue is provoked into pain and how long that pain takes to settle. A highly irritable condition needs gentler assessment and treatment techniques, while a low irritability condition can tolerate more aggressive testing and loading.

#9 · Assessment and Diagnosis

What does prognosis mean in a physiotherapy assessment?

Answer

Prognosis is the predicted level of function a client is expected to reach and the amount of time and number of visits needed to get there. It is based on the diagnosis, severity, chronicity, comorbidities, and the client's own goals and resources.

#10 · Assessment and Diagnosis

Why is reassessment performed within a single treatment session?

Answer

Reassessment within a session, often called an asterisk sign check, compares a key finding before and after an intervention to see if it changed. This tells the therapist whether the chosen treatment technique is helping, has no effect, or is making the problem worse.

#11 · Assessment and Diagnosis

What is an asterisk sign?

Answer

An asterisk sign is a specific, reproducible finding chosen during assessment, such as a painful movement, a positive special test, or a functional limitation, that the therapist marks to retest after treatment. It gives an objective marker of change and helps justify whether the plan is working.

#12 · Assessment and Diagnosis

What is the difference between subjective and objective assessment?

Answer

The subjective assessment is the history the client reports in their own words, including symptoms, onset, and functional impact. The objective assessment is the therapist's hands-on and observational findings, such as range of motion, strength, and special tests, gathered afterward to confirm or refute hypotheses from the history.

#13 · Assessment and Diagnosis

What is the purpose of the 24-hour symptom pattern question?

Answer

Asking how symptoms behave over 24 hours, including morning stiffness, night pain, and end-of-day fatigue, helps the therapist judge irritability and screen for inflammatory or serious pathology. Constant unremitting night pain that is unrelated to position is a red flag for possible malignancy or infection.

#14 · Assessment and Diagnosis

What is a body chart used for in assessment?

Answer

A body chart is a diagram the client marks to show the exact location, distribution, and type of symptoms, such as sharp pain, numbness, or pins and needles. It helps the therapist identify patterns that suggest a specific structure, nerve root, or peripheral nerve involvement.

#15 · Assessment and Diagnosis

What does SINSS stand for in subjective assessment?

Answer

SINSS stands for Severity, Irritability, Nature, Stage, and Stability. Therapists use it to judge how aggressively to test and treat a condition, with high severity or irritability meaning gentler handling is needed.

#16 · Assessment and Diagnosis

A client has a positive Lachman test. What does this indicate?

Answer

A positive Lachman test shows excessive anterior tibial translation with a soft or absent end feel when the knee is tested near 20 to 30 degrees of flexion. This suggests a tear of the anterior cruciate ligament and is considered more sensitive than the anterior drawer test.

#17 · Assessment and Diagnosis

What does a positive posterior drawer test suggest?

Answer

A positive posterior drawer test shows excessive posterior displacement of the tibia relative to the femur when the knee is flexed to 90 degrees and pushed backward. This indicates a posterior cruciate ligament tear.

#18 · Assessment and Diagnosis

Describe the McMurray test and its positive finding.

Answer

The McMurray test is performed with the knee fully flexed, then the tibia is rotated while the knee is extended, applying valgus stress for the medial meniscus or varus stress for the lateral meniscus. A palpable or audible click or clunk with reproduced pain suggests a meniscal tear.

#19 · Assessment and Diagnosis

What is the Thessaly test used to assess?

Answer

The Thessaly test assesses meniscal injury by having the client stand on one leg, supported by the examiner, and rotate the body while the knee is slightly flexed. Joint line pain, catching, or locking during rotation is a positive finding suggesting a meniscal tear.

#20 · Assessment and Diagnosis

What does a positive Neer impingement test indicate?

Answer

In the Neer test, the examiner passively flexes the client's arm overhead while stabilizing the scapula, which compresses the supraspinatus tendon against the acromion. Reproduction of pain suggests subacromial impingement.

#21 · Assessment and Diagnosis

Describe the Hawkins-Kennedy test.

Answer

The Hawkins-Kennedy test is performed by flexing the shoulder to 90 degrees, flexing the elbow to 90 degrees, and then internally rotating the shoulder. Pain during internal rotation suggests subacromial impingement, since this position compresses the rotator cuff tendons under the coracoacromial arch.

#22 · Assessment and Diagnosis

What is the empty can test used to assess and what is a positive finding?

Answer

The empty can test, also called the Jobe test, has the client resist downward pressure with the arm in 90 degrees of abduction, 30 degrees of horizontal adduction, and internal rotation as if emptying a can. Weakness or pain suggests supraspinatus tendon pathology or a tear.

#23 · Assessment and Diagnosis

What does a positive drop arm test suggest?

Answer

In the drop arm test, the client's arm is passively abducted to about 90 degrees and asked to slowly lower it. Inability to control the descent, with the arm dropping suddenly or with significant pain, suggests a full-thickness rotator cuff tear.

#24 · Assessment and Diagnosis

What is Speed's test used for?

Answer

Speed's test is performed by resisting shoulder flexion with the elbow extended and the forearm supinated. Pain in the bicipital groove suggests bicipital tendinopathy or labral involvement.

#25 · Assessment and Diagnosis

What does the apprehension test assess in the shoulder?

Answer

The apprehension test places the shoulder in 90 degrees of abduction and external rotation to simulate the position of anterior dislocation. A guarding or apprehensive response, rather than pain alone, suggests anterior shoulder instability.

#26 · Assessment and Diagnosis

What is Tinel's sign and where is it commonly tested?

Answer

Tinel's sign is elicited by tapping over a nerve, most commonly the median nerve at the wrist or the ulnar nerve at the elbow. A tingling or electric sensation distal to the tapping site suggests nerve irritation or regeneration at that point.

#27 · Assessment and Diagnosis

Describe Phalen's test and its positive finding.

Answer

Phalen's test has the client hold both wrists in full flexion, back to back, for up to one minute. Reproduction of numbness or tingling in the median nerve distribution suggests carpal tunnel syndrome.

#28 · Assessment and Diagnosis

What does a positive Finkelstein test indicate?

Answer

In Finkelstein's test the client makes a fist over the flexed thumb while the examiner deviates the wrist into ulnar deviation. Sharp pain over the radial styloid suggests De Quervain's tenosynovitis affecting the abductor pollicis longus and extensor pollicis brevis tendons.

#29 · Assessment and Diagnosis

What is the FABER test used to screen?

Answer

The FABER test, also called Patrick's test, places the hip in flexion, abduction, and external rotation with the foot resting on the opposite knee. Pain in the groin suggests hip joint pathology, while pain in the posterior sacroiliac region suggests sacroiliac joint dysfunction.

#30 · Assessment and Diagnosis

What does a positive Thomas test reveal?

Answer

The Thomas test has the client lie supine while one hip is pulled to the chest, and the tested leg is observed for its ability to stay flat on the table. If the tested thigh lifts off the table, it indicates a hip flexor contracture, most commonly of the iliopsoas or rectus femoris.

#31 · Assessment and Diagnosis

What is Ober's test used to assess?

Answer

Ober's test is performed side lying with the hip extended and slightly abducted, then the leg is allowed to lower toward the table. If the leg remains abducted and does not lower to the table, it suggests tightness of the iliotibial band and tensor fasciae latae.

#32 · Assessment and Diagnosis

Describe the straight leg raise test and its positive sign.

Answer

The straight leg raise test involves passively lifting the client's extended leg while they lie supine, watching for reproduction of radicular leg pain between about 30 and 70 degrees. A positive result suggests lumbar nerve root irritation or disc herniation affecting the sciatic nerve.

#33 · Assessment and Diagnosis

What is the slump test used to assess?

Answer

The slump test is a neurodynamic test done sitting, where the client slumps the spine, flexes the neck, and extends the knee while the ankle is dorsiflexed. Reproduction of familiar leg symptoms that ease with neck extension suggests neural tension affecting the sciatic nerve or lumbar nerve roots.

#34 · Assessment and Diagnosis

What does a positive Spurling test suggest?

Answer

Spurling's test involves extending, laterally flexing, and gently compressing the cervical spine toward the symptomatic side. Reproduction of radicular arm pain suggests cervical nerve root compression at that level.

#35 · Assessment and Diagnosis

What is the purpose of the cervical distraction test?

Answer

The cervical distraction test applies gentle upward traction to the head while the client is supine or seated. Relief of radicular arm symptoms supports a diagnosis of cervical nerve root compression, since decompression reduces pressure on the affected root.

#36 · Assessment and Diagnosis

What does the anterior drawer test at the ankle assess?

Answer

The anterior drawer test at the ankle stabilizes the tibia while pulling the calcaneus and talus forward. Excessive anterior translation compared to the other side suggests a tear of the anterior talofibular ligament.

#37 · Assessment and Diagnosis

What is the talar tilt test used to assess?

Answer

The talar tilt test applies an inversion force to the talus within the ankle mortise. Excessive inversion movement compared to the uninjured side suggests injury to the calcaneofibular ligament.

#38 · Assessment and Diagnosis

Describe Tinel's percussion test at the tarsal tunnel.

Answer

Tapping over the tarsal tunnel behind the medial malleolus, where the tibial nerve passes, reproduces tingling or shooting pain into the sole of the foot when positive. This supports a diagnosis of tarsal tunnel syndrome.

#39 · Assessment and Diagnosis

What is Tinetti's or Thompson's test used for at the ankle?

Answer

The Thompson test is performed by squeezing the calf while the client lies prone with the foot hanging off the table. Absence of passive plantarflexion of the foot suggests a complete rupture of the Achilles tendon.

#40 · Assessment and Diagnosis

What does a positive Tinel or valgus stress test at the elbow suggest?

Answer

The valgus stress test at the elbow is applied at about 20 to 30 degrees of flexion to unlock the olecranon. Pain or excessive medial gapping suggests injury to the ulnar collateral ligament, commonly seen in throwing athletes.

#41 · Assessment and Diagnosis

What is the normal active range of motion for shoulder flexion?

Answer

Normal shoulder flexion is approximately 180 degrees. This motion is measured with the goniometer axis at the lateral aspect of the acromion, the stationary arm aligned with the midaxillary line of the trunk, and the moving arm along the humerus.

#42 · Assessment and Diagnosis

What is the normal range of shoulder abduction?

Answer

Normal shoulder abduction is approximately 180 degrees, though the last portion of range requires scapular upward rotation and forearm supination to clear the greater tuberosity from the acromion. This combined movement is sometimes referred to as the scapulohumeral rhythm.

#43 · Assessment and Diagnosis

What is the normal range of elbow flexion and extension?

Answer

Normal elbow flexion is approximately 145 to 150 degrees, and normal extension is 0 degrees, with some individuals having a few degrees of hyperextension. The goniometer axis is placed at the lateral epicondyle for this measurement.

#44 · Assessment and Diagnosis

What is the normal range of knee flexion?

Answer

Normal knee flexion is approximately 135 to 140 degrees, while normal knee extension is 0 degrees. The goniometer axis is centered over the lateral epicondyle of the femur.

#45 · Assessment and Diagnosis

What is the normal range of hip flexion?

Answer

Normal hip flexion is approximately 120 degrees with the knee flexed, since a flexed knee relaxes the hamstrings and allows greater range. The goniometer axis is placed over the greater trochanter.

#46 · Assessment and Diagnosis

What is the normal range of ankle dorsiflexion and plantarflexion?

Answer

Normal ankle dorsiflexion is approximately 20 degrees and normal plantarflexion is approximately 50 degrees. These are measured with the goniometer axis over the lateral malleolus.

#47 · Assessment and Diagnosis

What is the normal range of cervical rotation?

Answer

Normal cervical rotation is approximately 80 degrees to each side. This can be quickly estimated clinically since most people can rotate the chin close to being in line with the shoulder.

#48 · Assessment and Diagnosis

What is the normal range of lumbar flexion?

Answer

Normal lumbar flexion is approximately 40 to 60 degrees, often measured functionally using the modified Schober test rather than a standard goniometer. The Schober test marks two points on the spine and measures how much the distance between them increases with forward bending.

#49 · Assessment and Diagnosis

What is the normal range of forearm pronation and supination?

Answer

Normal forearm pronation and supination are each approximately 80 to 90 degrees. Measurement is typically done with the elbow flexed to 90 degrees and tucked against the body to prevent shoulder substitution.

#50 · Assessment and Diagnosis

What is the normal range of wrist flexion and extension?

Answer

Normal wrist flexion is approximately 80 degrees and normal wrist extension is approximately 70 degrees. The goniometer axis is placed over the lateral aspect of the wrist near the triquetrum.

#51 · Assessment and Diagnosis

What is the normal range of hip internal and external rotation?

Answer

Normal hip internal rotation is approximately 35 to 45 degrees and external rotation is approximately 45 degrees, usually measured seated with the knee flexed to 90 degrees. Limited internal rotation is an early clinical sign of hip osteoarthritis.

#52 · Assessment and Diagnosis

What end feel is normal for elbow extension?

Answer

The normal end feel for elbow extension is bony, due to the olecranon process contacting the olecranon fossa. A different end feel, such as boggy or empty, at the end of extension suggests joint pathology or acute injury.

#53 · Assessment and Diagnosis

What end feel is normal for knee flexion?

Answer

The normal end feel for knee flexion is soft, due to the contact of the posterior thigh and calf muscle bulk. A firm or bony end feel earlier in range suggests an abnormal restriction such as meniscal blockage or joint effusion.

#54 · Assessment and Diagnosis

What is a capsular pattern and give an example for the shoulder.

Answer

A capsular pattern is a characteristic proportional limitation of motion caused by tightening of the whole joint capsule rather than one specific structure. For the glenohumeral joint the capsular pattern is external rotation most limited, followed by abduction, then internal rotation.

#55 · Assessment and Diagnosis

What does a non-capsular pattern of restriction suggest?

Answer

A non-capsular pattern is a restriction that does not follow the joint's typical capsular proportion, such as one motion being limited while others remain full. This suggests a localized problem like a loose body, ligament adhesion, or internal derangement rather than generalized capsulitis.

#56 · Assessment and Diagnosis

What does a manual muscle test grade of 3 out of 5 mean?

Answer

A grade of 3 means the client can move the limb through the full available range against gravity but cannot tolerate any additional manual resistance. It is the threshold grade separating antigravity strength from gravity-eliminated strength.

#57 · Assessment and Diagnosis

What does a manual muscle test grade of 2 out of 5 mean?

Answer

A grade of 2 means the client can move the limb through full range only when gravity is eliminated, such as sliding along a supported surface. It indicates significant weakness that cannot yet overcome the weight of the limb itself.

#58 · Assessment and Diagnosis

What does a manual muscle test grade of 1 out of 5 mean?

Answer

A grade of 1 means a palpable or visible flicker of muscle contraction is present but produces no actual joint movement. This is used to distinguish complete paralysis, grade 0, from a very early return of muscle activity.

#59 · Assessment and Diagnosis

What does a manual muscle test grade of 4 out of 5 mean?

Answer

A grade of 4 means the client can move through full range against gravity and tolerates moderate manual resistance but is weaker than the expected normal strength for that muscle. This grade often requires clinical judgment and comparison to the uninvolved side.

#60 · Assessment and Diagnosis

Why is manual muscle testing considered less objective in the 4 to 5 range?

Answer

The difference between a strong grade 4 and a full grade 5 depends heavily on the examiner's own strength and subjective sense of resistance, making it harder to standardize. Handheld dynamometry is often used clinically to get a more objective, reproducible strength value in this higher range.

#61 · Assessment and Diagnosis

What position is used to manually test the middle trapezius?

Answer

The middle trapezius is tested prone with the shoulder abducted to 90 degrees and externally rotated, thumb pointing toward the ceiling. Resistance is applied downward toward the floor while watching for scapular winging or substitution from other muscles.

#62 · Assessment and Diagnosis

What position is used to manually test the gluteus medius?

Answer

The gluteus medius is tested in side lying with the hip in slight extension and abduction, and the examiner resists hip abduction. Weakness of this muscle can produce a positive Trendelenburg sign during gait or single leg stance.

#63 · Assessment and Diagnosis

What does a positive Trendelenburg sign indicate?

Answer

A positive Trendelenburg sign is seen when the pelvis on the non-weight-bearing side drops during single leg stance on the opposite side. It indicates weakness or inhibition of the hip abductors, most commonly the gluteus medius, on the stance leg.

#64 · Assessment and Diagnosis

What position is used to manually test the quadriceps femoris?

Answer

The quadriceps is tested sitting with the knee flexed over the edge of a table, and the examiner resists knee extension near terminal extension. This position isolates the quadriceps while minimizing hip flexor contribution.

#65 · Assessment and Diagnosis

What is a break test in manual muscle testing?

Answer

A break test is when the examiner asks the client to hold a fixed position and then applies enough force to try to break, or overcome, the muscle's hold. It is contrasted with a make test, where the client actively pushes into the examiner's stationary resistance.

#66 · Assessment and Diagnosis

Which dermatome supplies the lateral aspect of the foot and small toe?

Answer

The S1 dermatome supplies sensation to the lateral foot and the fifth toe. It is commonly tested alongside the S1 myotome, ankle plantarflexion and eversion, and the Achilles reflex, since an S1 nerve root problem tends to affect all three together.

#67 · Assessment and Diagnosis

Which dermatome supplies the medial aspect of the lower leg and ankle?

Answer

The L4 dermatome supplies the medial lower leg and medial malleolus. It is tested alongside the L4 myotome, knee extension or ankle dorsiflexion depending on the reference, and the patellar reflex.

#68 · Assessment and Diagnosis

Which myotome is tested by resisted knee extension?

Answer

Resisted knee extension primarily tests the L3 myotome, with contribution from L4. Weakness here, especially combined with an absent patellar reflex, suggests an upper lumbar nerve root lesion.

#69 · Assessment and Diagnosis

Which myotome is tested by resisted great toe extension?

Answer

Resisted great toe extension tests the L5 myotome. It is one of the most sensitive single tests for an L5 radiculopathy since there is no strong reflex associated with this level to help confirm findings.

#70 · Assessment and Diagnosis

Which reflex is diminished with an L4 nerve root lesion?

Answer

The patellar tendon reflex, tested by tapping the patellar tendon with the knee relaxed in flexion, is primarily mediated by the L3 and L4 nerve roots. A diminished or absent patellar reflex supports an L3 or L4 radiculopathy.

#71 · Assessment and Diagnosis

Which reflex is diminished with an S1 nerve root lesion?

Answer

The Achilles tendon reflex, tested by tapping the Achilles tendon with the ankle in slight dorsiflexion, is mediated by the S1 and S2 nerve roots. A diminished or absent ankle reflex supports an S1 radiculopathy.

#72 · Assessment and Diagnosis

What does an upgoing Babinski sign indicate?

Answer

The Babinski sign is elicited by stroking the lateral sole of the foot from heel to toe. An abnormal response of great toe extension with fanning of the other toes indicates an upper motor neuron lesion in an adult, since this reflex is normally suppressed after infancy.

#73 · Assessment and Diagnosis

What is clonus and what does it indicate?

Answer

Clonus is a rhythmic, involuntary series of muscle contractions elicited by a quick sustained stretch, most often tested at the ankle by rapid dorsiflexion. Sustained clonus, meaning more than a few beats, indicates an upper motor neuron lesion.

#74 · Assessment and Diagnosis

List three signs of an upper motor neuron lesion.

Answer

Upper motor neuron lesions typically produce hypertonia or spasticity, hyperreflexia, and a positive Babinski sign. Weakness tends to be more generalized rather than following a single peripheral nerve or root pattern.

#75 · Assessment and Diagnosis

List three signs of a lower motor neuron lesion.

Answer

Lower motor neuron lesions typically produce hypotonia or flaccidity, hyporeflexia or absent reflexes, and muscle atrophy with fasciculations. Weakness usually follows the specific distribution of the affected nerve root or peripheral nerve.

#76 · Assessment and Diagnosis

What is the purpose of testing light touch, pinprick, and proprioception in a neurological screen?

Answer

Light touch and pinprick screen the spinothalamic and dorsal column sensory pathways for dermatomal or peripheral nerve deficits. Proprioception, often tested by moving the great toe up or down with eyes closed, screens the dorsal column pathway and balance-related sensory input.

#77 · Assessment and Diagnosis

What is the significance of a positive Hoffmann sign?

Answer

The Hoffmann sign is elicited by flicking the nail of the middle finger downward and watching for reflexive flexion of the thumb and index finger. A positive response suggests cervical spinal cord compression or another upper motor neuron lesion affecting the cervical cord.

#78 · Assessment and Diagnosis

What does the vertebral artery test screen for?

Answer

The vertebral artery test involves sustained cervical extension and rotation to assess for symptoms of vertebrobasilar insufficiency, such as dizziness, nystagmus, visual disturbance, or nausea. A positive test is a contraindication to cervical manipulation and high velocity techniques.

#79 · Assessment and Diagnosis

What is the alar ligament stress test used to assess?

Answer

The alar ligament stress test assesses upper cervical stability by side flexing or rotating the head while palpating the spinous process of C2 for movement. Excessive or absent movement suggests instability, which is a precaution before manual cervical techniques.

#80 · Assessment and Diagnosis

What is a myotome versus a dermatome?

Answer

A myotome is the group of muscles innervated by a single spinal nerve root, tested with resisted isometric contraction. A dermatome is the area of skin supplied by sensory fibers from a single spinal nerve root, tested with light touch or pinprick.

#81 · Assessment and Diagnosis

How is nerve conduction differentiated from a nerve root problem on physical exam?

Answer

A peripheral nerve lesion produces sensory and motor findings that follow the specific nerve's distribution, which may cross multiple dermatomes or myotomes. A nerve root lesion follows the segmental dermatome and myotome pattern instead, which is why testing both patterns together helps localize the lesion.

#82 · Assessment and Diagnosis

What is the significance of testing the biceps and triceps reflexes together?

Answer

The biceps reflex is mediated mainly by C5 and C6, while the triceps reflex is mediated mainly by C7. Comparing both helps localize a suspected cervical radiculopathy to a specific level when combined with matching myotome weakness.

#83 · Assessment and Diagnosis

What does a positive Romberg test suggest?

Answer

The Romberg test has the client stand with feet together and eyes closed, removing visual input to balance. A significant increase in sway or loss of balance suggests a proprioceptive or vestibular deficit rather than a cerebellar problem, since cerebellar ataxia is usually present with eyes open as well.

#84 · Assessment and Diagnosis

What is finger to nose testing used to assess?

Answer

Finger to nose testing asks the client to alternately touch their own nose and the examiner's finger. Overshooting, tremor, or inaccuracy, known as dysmetria, suggests cerebellar dysfunction.

#85 · Assessment and Diagnosis

What does rapid alternating movement testing assess?

Answer

Rapid alternating movement testing, such as quickly pronating and supinating the forearm, assesses coordination controlled by the cerebellum. Slow, clumsy, or irregular movement, called dysdiadochokinesia, suggests cerebellar involvement.

#86 · Assessment and Diagnosis

What is the normal resting heart rate range for a healthy adult?

Answer

The normal resting heart rate for a healthy adult is approximately 60 to 100 beats per minute. A well-conditioned athlete may have a lower resting rate due to greater stroke volume and cardiovascular efficiency.

#87 · Assessment and Diagnosis

What is the normal resting respiratory rate for a healthy adult?

Answer

The normal resting respiratory rate for a healthy adult is approximately 12 to 20 breaths per minute. A rate outside this range at rest, especially with signs of distress, warrants further monitoring and possible referral.

#88 · Assessment and Diagnosis

What is considered a normal blood pressure reading?

Answer

A normal blood pressure reading is generally considered below about 120 over 80 millimeters of mercury. Readings persistently at or above 130 over 80 are considered elevated or hypertensive and should be documented and monitored during exercise.

#89 · Assessment and Diagnosis

What is the normal range for oxygen saturation measured by pulse oximetry?

Answer

Normal oxygen saturation is typically 95 to 100 percent in a healthy adult at rest. A saturation below 90 percent is considered a sign of significant hypoxemia and generally requires urgent medical attention.

#90 · Assessment and Diagnosis

How is the Borg Rating of Perceived Exertion scale used?

Answer

The Borg scale, most commonly the 6 to 20 version, asks a client to rate how hard they feel they are working during exercise. It is used to guide exercise intensity safely, especially in clients who cannot use heart rate targets due to medication like beta blockers.

#91 · Assessment and Diagnosis

What is auscultated during a basic cardiorespiratory chest assessment?

Answer

Auscultation involves listening with a stethoscope over defined lung fields for normal versus adventitious breath sounds, such as crackles, wheezes, or diminished sounds, and over the heart for normal versus abnormal heart sounds. Findings help identify areas of reduced ventilation, fluid, or airway narrowing.

#92 · Assessment and Diagnosis

What does the term orthopnea mean?

Answer

Orthopnea is difficulty breathing while lying flat that improves when sitting or standing upright. It is commonly associated with congestive heart failure and is an important symptom to ask about during a cardiorespiratory history.

#93 · Assessment and Diagnosis

What is the significance of clubbing of the fingernails?

Answer

Clubbing is a bulbous enlargement and increased curvature of the nail bed, often associated with chronic hypoxia from cardiopulmonary disease such as chronic obstructive pulmonary disease or congenital heart disease. It is an observational finding that should prompt further medical investigation if newly noticed.

#94 · Assessment and Diagnosis

What is accessory muscle use a sign of during a respiratory assessment?

Answer

Accessory muscle use, such as visible activity of the sternocleidomastoid or scalene muscles during quiet breathing, suggests increased work of breathing. It is often seen in clients with significant respiratory distress or chronic obstructive lung disease.

#95 · Assessment and Diagnosis

What is the six-minute walk test used to measure?

Answer

The six-minute walk test measures the total distance a client can walk in six minutes at their own pace, used as a submaximal measure of functional exercise capacity. It is commonly used in cardiac and pulmonary rehabilitation and can be compared to age and sex predicted normal values.

#96 · Assessment and Diagnosis

How is oxygen desaturation with exertion interpreted?

Answer

A drop in oxygen saturation of 4 percent or more, or a value falling below 90 percent, during activity suggests exercise induced hypoxemia. This finding often warrants supplemental oxygen or activity modification and communication with the referring physician.

#97 · Assessment and Diagnosis

What is the purpose of monitoring heart rate during exercise testing?

Answer

Monitoring heart rate lets the therapist compare the client's response against predicted maximum heart rate and target training zones, and detect an abnormal or exaggerated response to exertion. An unexpectedly high or low heart rate response can indicate cardiovascular compromise or medication effects.

#98 · Assessment and Diagnosis

What does the term paroxysmal nocturnal dyspnea describe?

Answer

Paroxysmal nocturnal dyspnea is sudden shortness of breath that wakes a person from sleep, usually relieved by sitting upright. It is a classic symptom associated with left-sided heart failure and should be documented carefully in a cardiorespiratory history.

#99 · Assessment and Diagnosis

What is auscultated to differentiate a wheeze from a crackle?

Answer

A wheeze is a continuous, high-pitched musical sound caused by airflow through narrowed airways, often heard in asthma or chronic obstructive pulmonary disease. A crackle is a discontinuous, popping sound often caused by fluid or the sudden opening of collapsed airways, commonly heard in pneumonia or heart failure.

#100 · Assessment and Diagnosis

What does pursed lip breathing observed on assessment suggest?

Answer

Pursed lip breathing is a compensatory strategy where a client exhales through partly closed lips to slow airflow and keep airways open longer. It is commonly seen in clients with chronic obstructive pulmonary disease to reduce dynamic airway collapse and improve gas exchange.

#101 · Assessment and Diagnosis

Name three red flags for cauda equina syndrome.

Answer

Red flags for cauda equina syndrome include saddle anesthesia, new bladder or bowel dysfunction such as retention or incontinence, and bilateral leg weakness or sensory loss. Any of these findings requires immediate referral to emergency care since delayed treatment can cause permanent neurological damage.

#102 · Assessment and Diagnosis

What history findings would raise suspicion of spinal malignancy?

Answer

Red flags for possible spinal malignancy include unexplained weight loss, a history of cancer, age over 50, pain that is constant and unremitting regardless of position, and pain that is worse at night. These findings should prompt referral for further medical investigation rather than continuing routine physiotherapy treatment alone.

#103 · Assessment and Diagnosis

What are red flags for a possible spinal fracture?

Answer

Red flags for spinal fracture include a history of significant trauma, prolonged corticosteroid use, osteoporosis, and older age with even minor trauma. Point tenderness over a vertebra combined with these risk factors should raise clinical suspicion and prompt imaging referral.

#104 · Assessment and Diagnosis

What are signs suggestive of a spinal infection such as discitis or osteomyelitis?

Answer

Signs suggestive of spinal infection include fever, chills, unexplained weight loss, recent infection or surgery, immunosuppression, and constant severe pain unrelieved by rest or position. These systemic signs combined with spinal pain warrant urgent medical referral.

#105 · Assessment and Diagnosis

What vital sign or symptom combination suggests deep vein thrombosis in the calf?

Answer

Unilateral calf swelling, warmth, redness, and tenderness, sometimes with pain on passive ankle dorsiflexion, raise suspicion for deep vein thrombosis. Because of the risk of pulmonary embolism if a clot dislodges, this presentation requires urgent medical referral rather than manual therapy to the leg.

#106 · Assessment and Diagnosis

What are red flags suggesting abdominal aortic aneurysm as a source of back pain?

Answer

Red flags for abdominal aortic aneurysm include a pulsatile abdominal mass, unrelenting back or abdominal pain that does not change with position or movement, and a history of vascular disease. This presentation, particularly in older adults, requires urgent medical evaluation.

#107 · Assessment and Diagnosis

What symptoms during cervical spine assessment suggest possible vertebrobasilar insufficiency?

Answer

The 5 D's and 3 N's are used as a memory aid: dizziness, diplopia, dysarthria, dysphagia, drop attacks, nausea, nystagmus, and numbness of the face. Any of these symptoms during cervical movement or the vertebral artery test should stop further mechanical testing and prompt medical referral.

#108 · Assessment and Diagnosis

What chest symptoms during a physiotherapy session require immediate medical attention?

Answer

Chest pain or pressure, especially radiating to the jaw, arm, or back, along with shortness of breath, sweating, or nausea, are classic warning signs of a possible cardiac event. The therapist should stop the session and activate emergency medical services rather than continuing treatment.

#109 · Assessment and Diagnosis

What are red flags associated with cervical myelopathy?

Answer

Red flags for cervical myelopathy include gait disturbance, loss of hand dexterity, bilateral or diffuse limb symptoms, hyperreflexia, and a positive Hoffmann or Babinski sign. This presentation suggests spinal cord compression and requires urgent medical referral rather than routine manual therapy.

#110 · Assessment and Diagnosis

What symptom pattern following recent surgery or long travel raises suspicion for pulmonary embolism?

Answer

Sudden onset shortness of breath, sharp chest pain that worsens with breathing, and a rapid heart rate, particularly after recent surgery, immobility, or long travel, suggest a possible pulmonary embolism. This is a medical emergency requiring the therapist to stop treatment and seek immediate care.

#111 · Assessment and Diagnosis

What are red flags for possible fracture after an ankle injury that guide imaging referral?

Answer

The Ottawa Ankle Rules recommend an x-ray if there is bone tenderness at the posterior edge or tip of either malleolus, or if the client is unable to bear weight for four steps both immediately after injury and in the clinic. These rules help therapists decide when imaging is needed without over-referring every ankle sprain.

#112 · Assessment and Diagnosis

What history finding in a client with knee pain should raise suspicion for septic arthritis?

Answer

Fever, significant joint swelling, warmth, redness, and severe pain with any movement of the joint suggest septic arthritis. This presentation is a medical emergency because delayed treatment can rapidly destroy joint cartilage.

#113 · Assessment and Diagnosis

What are red flags for possible temporal arteritis in an older adult with headache?

Answer

Red flags for temporal arteritis include new onset headache in someone over age 50, jaw pain with chewing, scalp tenderness, and visual disturbance. This condition can cause sudden permanent vision loss if untreated, so it requires urgent medical referral.

#114 · Assessment and Diagnosis

What findings suggest a possible stress fracture rather than a simple muscle strain?

Answer

Point tenderness directly over bone, pain that worsens with weight-bearing activity and improves with rest, and a positive hop test or fulcrum test suggest a stress fracture rather than a soft tissue strain. Risk factors like rapid training increases, low bone density, or amenorrhea increase suspicion further.

#115 · Assessment and Diagnosis

Why do therapists screen for unexplained bruising or bleeding during assessment?

Answer

Unexplained or excessive bruising can be a sign of blood clotting disorders, anticoagulant medication effects, or in rare cases hematologic malignancy. It changes how aggressively manual therapy or exercise resistance can be safely applied and may warrant physician follow-up.

#116 · Assessment and Diagnosis

What does the Oswestry Disability Index measure?

Answer

The Oswestry Disability Index is a self-report questionnaire that measures the degree of functional limitation caused by low back pain across activities like personal care, lifting, walking, and sitting. It is widely used in Canadian practice to track progress and support outcome-based billing or reporting.

#117 · Assessment and Diagnosis

What does the Neck Disability Index measure?

Answer

The Neck Disability Index is a self-report questionnaire similar in structure to the Oswestry, measuring how neck pain limits daily activities such as concentration, driving, and sleeping. It is used to establish baseline disability and track change over a course of treatment for cervical conditions.

#118 · Assessment and Diagnosis

What is the DASH questionnaire used to assess?

Answer

The DASH, which stands for Disabilities of the Arm, Shoulder and Hand, is a self-report outcome measure assessing symptoms and function in the upper extremity regardless of the specific diagnosis. It is commonly used across shoulder, elbow, wrist, and hand conditions in Canadian clinics.

#119 · Assessment and Diagnosis

What is the Berg Balance Scale used to assess?

Answer

The Berg Balance Scale is a performance-based measure with 14 tasks, such as standing unsupported, reaching forward, and turning to look behind, scored from 0 to 4 each for a maximum of 56. It is widely used to assess fall risk and balance in older adults and neurological populations.

#120 · Assessment and Diagnosis

What score on the Berg Balance Scale is generally considered to indicate increased fall risk?

Answer

A total score at or below 45 out of 56 on the Berg Balance Scale is generally considered to indicate an increased risk of falling. Lower scores suggest greater balance impairment and the need for more targeted balance intervention.

#121 · Assessment and Diagnosis

What is the Timed Up and Go test and how is it performed?

Answer

The Timed Up and Go test measures the time it takes a client to rise from a standard chair, walk three meters, turn around, walk back, and sit down again. It is a quick screening tool for mobility and fall risk, commonly used with older adults.

#122 · Assessment and Diagnosis

What Timed Up and Go score is generally associated with increased fall risk?

Answer

A time of 12 seconds or longer to complete the Timed Up and Go test is generally associated with increased fall risk in older adults. Faster times generally reflect better functional mobility.

#123 · Assessment and Diagnosis

What is the Numeric Pain Rating Scale?

Answer

The Numeric Pain Rating Scale asks the client to rate their pain on a scale from 0, meaning no pain, to 10, meaning the worst pain imaginable. It is a simple, quick, and widely used tool to track pain intensity changes across a treatment episode.

#124 · Assessment and Diagnosis

What is the Patient-Specific Functional Scale used for?

Answer

The Patient-Specific Functional Scale asks the client to identify three to five important activities they have difficulty with due to their condition and rate their ability to perform each on a 0 to 10 scale. It is individualized to the client and is widely used across many conditions in Canadian outpatient practice.

#125 · Assessment and Diagnosis

What does the Lower Extremity Functional Scale measure?

Answer

The Lower Extremity Functional Scale is a 20-item self-report questionnaire measuring difficulty with functional tasks like walking, climbing stairs, and getting in and out of a car due to lower limb impairment. It is used broadly across hip, knee, ankle, and foot conditions.

#126 · Assessment and Diagnosis

What is the minimal clinically important difference, or MCID?

Answer

The minimal clinically important difference is the smallest change in an outcome measure score that a client would perceive as meaningful, not just statistically detectable. It helps a therapist judge whether a measured improvement actually represents a real functional benefit.

#127 · Assessment and Diagnosis

What is the KOOS questionnaire used to assess?

Answer

The KOOS, or Knee injury and Osteoarthritis Outcome Score, assesses symptoms, pain, function in daily living, function in sport and recreation, and knee-related quality of life. It is commonly used for clients with knee osteoarthritis or ligament injuries.

#128 · Assessment and Diagnosis

What does the term reliability mean when evaluating an outcome measure?

Answer

Reliability refers to how consistent or reproducible a measure's results are when repeated under the same conditions, whether by the same tester or different testers. A highly reliable measure gives similar results each time if the client's true status has not changed.

#129 · Assessment and Diagnosis

What does the term validity mean when evaluating an outcome measure?

Answer

Validity refers to whether a measure actually assesses what it claims to assess. A goniometer reading has strong validity for measuring joint range of motion, but would have poor validity if used to try to measure pain intensity.

#130 · Assessment and Diagnosis

What is the difference between sensitivity and specificity of a special test?

Answer

Sensitivity is how well a test correctly identifies people who truly have the condition, meaning a highly sensitive negative test helps rule the condition out. Specificity is how well a test correctly identifies people who truly do not have the condition, meaning a highly specific positive test helps rule the condition in.

#131 · Assessment and Diagnosis

What is the mnemonic SnNout used for?

Answer

SnNout means that a test with high sensitivity, when negative, helps rule a condition out, since few true cases would be missed. This is useful for choosing screening tests early in an assessment to filter out unlikely diagnoses.

#132 · Assessment and Diagnosis

What is the mnemonic SpPin used for?

Answer

SpPin means that a test with high specificity, when positive, helps rule a condition in, since few false positives occur. This is useful for confirming a suspected diagnosis after screening has narrowed down the possibilities.

#133 · Assessment and Diagnosis

What is the Functional Independence Measure used to assess?

Answer

The Functional Independence Measure, often called the FIM, assesses the level of assistance a client needs across self-care, mobility, communication, and cognitive tasks, scored from total assistance to complete independence. It is widely used in Canadian inpatient rehabilitation settings to track progress and plan discharge.

#134 · Assessment and Diagnosis

What is the purpose of using standardized outcome measures in physiotherapy practice?

Answer

Standardized outcome measures give an objective, reproducible way to document a client's baseline status and track meaningful change over the course of care. They also support communication with other providers, funders, and third-party payers who require documented evidence of progress.

#135 · Assessment and Diagnosis

What is the six-minute walk test's relevance as an outcome measure beyond cardiorespiratory conditions?

Answer

Beyond cardiorespiratory populations, the six-minute walk test is also used with neurological and orthopedic clients to track functional endurance and walking capacity over time. Comparing distance walked before and after a treatment program provides an objective measure of functional improvement.

#136 · Assessment and Diagnosis

Why does a physiotherapy interview begin with open-ended questions?

Answer

Open-ended questions allow the client to describe their concerns in their own words without being led toward a specific answer, which reduces bias in the history taken. The therapist can follow up with more focused closed questions afterward to clarify specific details.

#137 · Assessment and Diagnosis

What is the purpose of asking about mechanism of injury during an interview?

Answer

The mechanism of injury describes exactly how an injury occurred, including the direction of force, position of the body, and any sounds or sensations felt at the time. This information helps the therapist generate hypotheses about which structures are most likely involved before any hands-on testing begins.

#138 · Assessment and Diagnosis

Why is a client's occupation relevant to a physiotherapy assessment?

Answer

A client's occupation reveals repetitive postures, loads, and movement demands that may contribute to the current condition and will need to be addressed for a safe return to work. It also helps set realistic and relevant functional goals tied to what the client actually needs to do daily.

#139 · Assessment and Diagnosis

Why do therapists ask about past medical history even for a straightforward musculoskeletal complaint?

Answer

Past medical history can reveal comorbidities, prior surgeries, or conditions like diabetes and osteoporosis that affect healing time, treatment safety, and overall prognosis. It can also uncover contraindications or precautions that change how aggressively the therapist can assess or treat.

#140 · Assessment and Diagnosis

What is meant by a client's goals in the context of assessment and treatment planning?

Answer

A client's goals describe what they personally want to achieve, such as returning to a specific sport, job task, or hobby, rather than only what the therapist observes as impaired. Building the plan around these goals increases motivation and makes the plan more relevant to the person's actual life.

#141 · Assessment and Diagnosis

What does observation of posture in standing contribute to the assessment?

Answer

Postural observation looks at alignment of the head, shoulders, spine, pelvis, and lower limbs for asymmetries such as a forward head posture, scoliosis, or pelvic obliquity. These findings help generate hypotheses about muscle imbalances or structural factors contributing to the client's symptoms.

#142 · Assessment and Diagnosis

What does gait observation add to a musculoskeletal assessment?

Answer

Gait observation looks at stride length, symmetry, weight-bearing patterns, and compensations such as a limp or reduced arm swing. Deviations can point toward pain avoidance, weakness, joint restriction, or a neurological issue that needs further specific testing.

#143 · Assessment and Diagnosis

Why do therapists palpate before and after performing special tests?

Answer

Palpation before testing establishes baseline tenderness, swelling, temperature, and tissue texture, giving context for interpreting later findings. Palpation after testing, particularly after a provocative special test, helps confirm whether the same structure reacted as expected.

#144 · Assessment and Diagnosis

What is the significance of unilateral versus bilateral swelling of the extremities in assessment?

Answer

Unilateral swelling more often suggests a localized problem such as injury, infection, or deep vein thrombosis in that specific limb. Bilateral swelling more often suggests a systemic cause such as heart failure, kidney disease, or venous insufficiency and may require medical referral.

#145 · Assessment and Diagnosis

What is the purpose of assessing skin integrity during a physiotherapy examination?

Answer

Assessing skin integrity checks for wounds, pressure injuries, rashes, or changes in color and temperature that could affect treatment choices, such as avoiding direct pressure over a fragile area. It is especially important in clients with diabetes, reduced sensation, or limited mobility who are at higher risk of skin breakdown.

#146 · Assessment and Diagnosis

Why might a therapist screen the abdomen during a lower back assessment?

Answer

Some visceral organs, such as the kidneys or gastrointestinal tract, can refer pain to the low back or flank, mimicking a musculoskeletal problem. Screening questions about digestion, urination, or menstrual cycle help the therapist decide if the symptoms may be coming from an internal organ rather than the spine or muscles.

#147 · Assessment and Diagnosis

What history findings would prompt screening for a genitourinary source of low back or pelvic pain?

Answer

Changes in urinary frequency, urgency, blood in the urine, or pain with urination alongside back or pelvic pain suggest a possible genitourinary cause such as a kidney stone or infection. These findings warrant referral back to the family physician rather than continued mechanical treatment alone.

#148 · Assessment and Diagnosis

Why is documenting baseline findings important before starting any treatment?

Answer

Baseline documentation records the client's exact starting point for pain, range of motion, strength, and function, which allows objective comparison later in the plan of care. Without clear baselines, it becomes difficult to prove progress or justify continuing, changing, or discharging from treatment.

#149 · Assessment and Diagnosis

What is meant by a working hypothesis in physiotherapy assessment?

Answer

A working hypothesis is the therapist's best current explanation for the client's presentation, formed from the subjective history and refined through objective testing. It is treated as provisional and is updated or discarded as new findings either support or contradict it during the assessment.

#150 · Assessment and Diagnosis

Why is ongoing reassessment throughout an entire episode of care considered essential, not optional?

Answer

Ongoing reassessment throughout the episode of care confirms whether the client is progressing as expected, whether the initial diagnosis still fits, and whether the plan needs to change. Without regular reassessment, a therapist could continue an ineffective or even harmful treatment far longer than necessary.

#151 · Care Planning

What does the S in a SMART goal stand for?

Answer

Specific. The goal must name the exact task, body part, or activity, not a vague outcome. For example, 'walk 100 meters with a rollator on level ground' is specific, while 'improve mobility' is not.

#152 · Care Planning

What does the M in a SMART goal stand for?

Answer

Measurable. The goal needs a number, scale, or test result attached so progress can be tracked objectively. A measurable goal lets the therapist compare baseline to reassessment using the same metric.

#153 · Care Planning

What does the A in a SMART goal stand for?

Answer

Achievable. The target must be realistic given the client's diagnosis, prognosis, and current impairments. Setting an unachievable goal wastes session time and can discourage the client.

#154 · Care Planning

What does the R in a SMART goal stand for?

Answer

Relevant. The goal should connect directly to the client's own priorities and daily function, not just an impairment the therapist finds interesting. A relevant goal keeps the plan client centered.

#155 · Care Planning

What does the T in a SMART goal stand for?

Answer

Time bound. Every goal needs a target date so the team knows when to reassess. Without a timeframe there is no way to judge whether progress is on track.

#156 · Care Planning

Write a SMART short-term goal for a post-op total knee replacement client at week 2.

Answer

Client will achieve active knee flexion of 90 degrees in supine within 2 weeks to allow safe transfer in and out of a car. This states the exact movement, the number, and the deadline, and ties it to a functional need.

#157 · Care Planning

Long-term goal versus short-term goal

Answer

A long-term goal describes the overall outcome expected by discharge, such as returning to work duties. Short-term goals are the smaller, sequential steps that build toward that outcome and are usually reassessed every 1 to 2 weeks.

#158 · Care Planning

Why should goals be written in client-centered language?

Answer

Client-centered goals reflect what matters to the person, such as playing with grandchildren or returning to a hobby, rather than only impairment numbers. This improves buy-in and adherence to the home program because the client sees the point of the work.

#159 · Care Planning

What is a baseline measurement used for in care planning?

Answer

A baseline is the objective starting value of a measure, such as range of motion, strength grade, or a functional outcome score, taken at initial assessment. All later goals and progress notes are compared against this starting point.

#160 · Care Planning

Client wants to 'get stronger' as a goal. What is the problem?

Answer

This statement is not specific or measurable because it does not name a muscle group, a strength grade, or a functional task. The therapist should rephrase it into something like increasing quadriceps strength from 3/5 to 4/5 to allow stair climbing without a handrail.

#161 · Care Planning

How often should goals typically be reviewed in an active treatment plan?

Answer

Goals are usually reviewed at each reassessment point, commonly every 2 to 4 weeks or at a set number of visits depending on the funding body. If progress is faster or slower than expected the goals and timeline are updated at that point.

#162 · Care Planning

What outcome measure fits a low back pain client's functional goal?

Answer

The Oswestry Disability Index or the Roland Morris Disability Questionnaire are common choices because they capture how pain limits daily activity. Either score gives a measurable number to attach to a SMART goal.

#163 · Care Planning

Term: prognosis in a physiotherapy plan

Answer

Prognosis is the predicted level of function and recovery timeline based on the diagnosis, the client's history, and known healing rates of tissue involved. It shapes how ambitious or conservative the goals and treatment plan should be.

#164 · Care Planning

What is the ICF framework and why does it matter for goal setting?

Answer

The International Classification of Functioning, Disability and Health organizes problems into body structure and function, activity, and participation levels. Using it helps the therapist set goals across all three levels instead of only fixing an impairment while ignoring participation in life roles.

#165 · Care Planning

Scenario: client with rotator cuff tendinopathy wants to return to overhead painting work. What level of ICF goal is this?

Answer

This is a participation level goal because it concerns a work role, not just shoulder range of motion. The plan should still include activity level goals, like reaching overhead without pain, that feed into this larger participation goal.

#166 · Care Planning

What is manual therapy in an MSK plan of care?

Answer

Manual therapy is a hands-on technique category that includes joint mobilization, manipulation, soft tissue mobilization, and stretching performed by the therapist. It is chosen to reduce pain, improve joint mobility, or reduce muscle guarding before active exercise is progressed.

#167 · Care Planning

Term: modalities in an MSK care plan

Answer

Modalities are physical agents such as heat, ice, ultrasound, or electrical stimulation that are used to manage pain or swelling for a short period. They are best used as a brief adjunct to support active treatment, not as the main long-term intervention.

#168 · Care Planning

Term: therapeutic exercise

Answer

Therapeutic exercise is any planned physical movement prescribed to restore or improve strength, range of motion, endurance, balance, or motor control. It is the core, active component of most rehabilitation plans and drives long-term change more than passive treatments.

#169 · Care Planning

Why do most evidence-based MSK plans favor active exercise over passive modalities?

Answer

Active exercise produces lasting changes in strength, motor control, and tissue tolerance, while passive modalities like heat or ultrasound mainly provide short-term symptom relief. Current guidelines recommend using passive treatments only as a brief adjunct to support participation in active exercise.

#170 · Care Planning

Scenario: client with acute lateral ankle sprain, 2 days post-injury, still swollen and painful with weight bearing. What is the priority intervention category?

Answer

Early management should focus on protection, relative rest, gentle range of motion, and edema control such as compression and elevation. Aggressive stretching or loading is delayed until swelling and pain settle enough to tolerate progressive loading.

#171 · Care Planning

What is graded exposure and when is it used?

Answer

Graded exposure is a behavioral approach where a client gradually performs feared or avoided movements in small, tolerable steps. It is used for clients with high fear avoidance, such as chronic low back pain, to rebuild confidence in movement rather than only targeting tissue impairment.

#172 · Care Planning

Term: isometric exercise and its common use in tendinopathy

Answer

Isometric exercise involves muscle contraction without joint movement, holding a fixed position under load. It is often used early in tendinopathy rehab because it can reduce pain and maintain strength without the shear stress of dynamic loading.

#173 · Care Planning

What is the general loading progression for tendinopathy rehabilitation?

Answer

The progression typically moves from isometric holds, to isotonic strengthening, to energy storage and release exercises such as hopping, and finally to sport-specific activity. Each stage is only advanced once the client tolerates the current load with acceptable pain response.

#174 · Care Planning

Scenario: client with chronic knee osteoarthritis, main goals are pain reduction and function. What core interventions should the plan include?

Answer

The plan should center on quadriceps and hip strengthening, aerobic exercise, and education on activity pacing and weight management if relevant. Modalities can be added briefly for symptom control but should not replace the exercise program.

#175 · Care Planning

Term: closed kinetic chain exercise

Answer

A closed kinetic chain exercise is one where the distal limb segment is fixed against a surface, such as a squat or leg press. These exercises are often favored early after knee ligament surgery because they create more joint compression and less shear force than open chain exercises.

#176 · Care Planning

Term: open kinetic chain exercise

Answer

An open kinetic chain exercise is one where the distal segment moves freely, such as a seated knee extension machine. These are often introduced later after ACL reconstruction because they can place higher shear stress on the healing graft, especially in the last 30 degrees of extension.

#177 · Care Planning

How does load progression typically follow the concept of progressive overload?

Answer

Progressive overload means gradually increasing the demand placed on tissue over time, through changes in resistance, repetitions, speed, or complexity. The plan should only add one variable at a time so the therapist can judge which change caused any change in symptoms.

#178 · Care Planning

Scenario: client reports 6 out of 10 pain that settles within 24 hours after each exercise session. What does this suggest for the plan?

Answer

This pain pattern is generally considered acceptable during tendon or joint rehabilitation as long as it resolves and does not worsen baseline symptoms. The current dose can likely continue or progress slightly, but pain that lasts beyond 24 hours signals the load should be reduced.

#179 · Care Planning

What criteria guide when to advance a strengthening exercise to the next stage?

Answer

Advancement is guided by the client meeting current strength or control benchmarks, having an acceptable pain response, and showing good movement quality without compensation. Time alone should not be the only factor used to decide progression.

#180 · Care Planning

Term: proprioceptive neuromuscular facilitation (PNF)

Answer

PNF is a set of stretching and strengthening techniques that use diagonal movement patterns paired with the body's own reflexes, such as contract-relax stretching. It is often chosen to improve flexibility or motor recruitment in both orthopedic and neurological populations.

#181 · Care Planning

Scenario: client with chronic non-specific low back pain and high fear of movement. Which intervention approach best fits current evidence?

Answer

A combination of graded activity, education about pain and reassurance about tissue safety, and general exercise is favored over passive treatment alone. The plan should target the psychosocial fear-avoidance pattern as much as the physical impairment.

#182 · Care Planning

Term: neuroplasticity as applied to stroke rehabilitation

Answer

Neuroplasticity is the brain's ability to reorganize connections and form new pathways in response to repeated practice and stimulation. Stroke rehab plans use high repetition, task-specific practice to drive this reorganization and support functional recovery.

#183 · Care Planning

What is task-specific training in neurological rehabilitation?

Answer

Task-specific training means practicing the actual functional task the client needs, such as standing up from a chair, rather than only isolated muscle exercises. This approach is supported because motor learning transfers best when practice closely matches the real activity.

#184 · Care Planning

Scenario: client 3 weeks post-stroke with dense hemiplegia and no active movement in the affected arm. What intervention focus fits this stage?

Answer

At this stage the plan should focus on positioning to prevent contracture, passive range of motion, and facilitation techniques to encourage any emerging voluntary movement. Constraint-induced movement therapy is not appropriate yet because it requires some active wrist and finger extension to start.

#185 · Care Planning

Term: constraint-induced movement therapy (CIMT)

Answer

CIMT restrains the less affected limb to force intensive, repeated use of the more affected limb after stroke. It requires the client to have a minimum amount of active movement already present in the wrist and fingers before it can be used.

#186 · Care Planning

Scenario: client with Parkinson disease reports freezing episodes when walking through doorways. What cueing strategy helps?

Answer

External cues such as visual floor markers, a metronome for rhythm, or verbal step commands can help override the freezing episode. These cues bypass the faulty automatic movement pathway by engaging conscious attention to movement.

#187 · Care Planning

Term: motor learning stages

Answer

Motor learning progresses through cognitive, associative, and autonomous stages. In the cognitive stage the client needs frequent feedback and conscious attention, while in the autonomous stage the movement becomes automatic and needs little conscious thought.

#188 · Care Planning

What type of feedback schedule best supports long-term motor learning retention?

Answer

Reduced or intermittent feedback, rather than feedback after every single trial, better supports long-term retention because it forces the learner to use internal error detection. Feedback given every trial can create dependence and weaker retention once the therapist is no longer present.

#189 · Care Planning

Scenario: client with incomplete spinal cord injury, goal is independent transfers. What should the plan prioritize?

Answer

The plan should prioritize upper body strengthening, trunk control, and repeated practice of the specific transfer technique the client will use daily. Equipment needs such as a transfer board should be assessed and trialed alongside the exercise progression.

#190 · Care Planning

Term: spasticity management in neuro rehab

Answer

Spasticity management can include stretching, positioning, splinting, and sometimes medication or botulinum toxin injections coordinated with the physician. Physiotherapy exercise should be timed relative to these medical treatments to take advantage of periods of reduced muscle tone.

#191 · Care Planning

Scenario: client with multiple sclerosis reports significant fatigue that limits exercise tolerance. How should the plan adapt?

Answer

The plan should use energy conservation principles, shorter exercise bouts with rest breaks, and monitoring for heat sensitivity since heat can worsen MS symptoms temporarily. Exercise intensity should be adjusted based on daily fatigue level rather than a fixed protocol.

#192 · Care Planning

Term: balance training progression

Answer

Balance training typically progresses from a stable surface with a wide base of support to unstable surfaces, narrow stances, and added dual tasks like talking while standing. Progression should only occur once the client is safe and steady at the current level.

#193 · Care Planning

Scenario: client with vestibular hypofunction and dizziness with head turns. What exercise type targets this?

Answer

Gaze stabilization exercises, such as repeated head turns while focusing on a fixed target, help the vestibular system adapt. These exercises work by provoking a controlled amount of symptoms to drive central compensation over repeated sessions.

#194 · Care Planning

What is airway clearance technique and when is it included in a plan?

Answer

Airway clearance techniques, such as huffing, active cycle of breathing, or postural drainage, help move and remove excess secretions from the lungs. They are included for clients with conditions like cystic fibrosis, bronchiectasis, or pneumonia with excess mucus production.

#195 · Care Planning

Term: active cycle of breathing technique (ACBT)

Answer

ACBT is a three-part breathing cycle that combines breathing control, thoracic expansion exercises, and the forced expiration technique. It helps loosen and clear secretions without relying on a caregiver or equipment, making it useful for independent home self-management.

#196 · Care Planning

Scenario: client with COPD, moderate dyspnea, deconditioned, referred for pulmonary rehab. What should the core plan include?

Answer

The plan should include supervised aerobic exercise, strength training for limb muscles, breathing technique education such as pursed lip breathing, and self-management education on recognizing exacerbations. Aerobic training intensity is usually guided by a dyspnea scale rather than heart rate alone in advanced disease.

#197 · Care Planning

Term: pursed lip breathing

Answer

Pursed lip breathing involves inhaling through the nose and exhaling slowly through pursed lips, which creates back pressure in the airways. This technique helps keep airways open longer during exhalation and can reduce the feeling of breathlessness in COPD.

#198 · Care Planning

What are the phases of cardiac rehabilitation?

Answer

Phase 1 is the inpatient stage right after a cardiac event, focused on early mobilization. Phase 2 is supervised outpatient exercise training with monitoring, and Phase 3 is a longer-term maintenance program with less direct supervision that supports lifelong activity habits.

#199 · Care Planning

Scenario: client 2 days post myocardial infarction, medically stable, in hospital. What is the appropriate intervention?

Answer

The appropriate intervention is Phase 1 cardiac rehab, which includes low-level mobilization such as sitting, standing, and short supervised walks with vital sign monitoring. Intensity is kept low and progression is guided by the client's response and physician clearance.

#200 · Care Planning

Term: rate of perceived exertion (RPE) and its use in cardiorespiratory plans

Answer

RPE is a subjective scale, commonly the Borg scale from 6 to 20, used to gauge exercise intensity based on how hard the activity feels. It is especially useful for monitoring intensity in clients on heart rate altering medications like beta blockers, where target heart rate zones are less reliable.

#201 · Care Planning

Scenario: client on beta blockers starting a cardiac rehab exercise program. Why is target heart rate a poor intensity guide here?

Answer

Beta blockers blunt the normal rise in heart rate during exercise, so heart rate no longer reflects true effort. RPE or a talk test is a more reliable way to set and monitor exercise intensity for this client.

#202 · Care Planning

Term: 6-minute walk test in cardiorespiratory care planning

Answer

The 6-minute walk test measures the total distance a client can walk in 6 minutes and is used as a functional aerobic capacity outcome measure. It is often chosen as a baseline and reassessment tool for COPD and cardiac rehab goals because it reflects real-world walking endurance.

#203 · Care Planning

Scenario: client with heart failure reports 3 pounds of weight gain overnight and increased shortness of breath. What should the plan include for self-management?

Answer

The plan should teach the client to monitor daily weight and recognize this as a sign of fluid retention that requires contacting their physician promptly. Exercise for that session should be held or modified until the client is medically reassessed.

#204 · Care Planning

Term: incentive spirometry

Answer

Incentive spirometry is a device-based breathing exercise where the client takes slow, deep breaths against visual feedback to encourage lung expansion. It is commonly used after abdominal or thoracic surgery to prevent atelectasis and pneumonia.

#205 · Care Planning

Scenario: post-abdominal surgery client with shallow breathing and reduced cough due to incision pain. What should the plan address first?

Answer

The plan should first address pain management strategies like splinting the incision with a pillow during coughing, along with deep breathing exercises and early mobilization. Effective pain control allows the client to participate fully in airway clearance and movement.

#206 · Care Planning

Term: home exercise program (HEP) dosage

Answer

HEP dosage refers to the specific sets, repetitions, frequency, and intensity prescribed for exercises to be done outside supervised sessions. Clear dosage instructions, not just a list of exercises, are what allow the client to safely and effectively self-manage.

#207 · Care Planning

Why is exercise progression built into a home program rather than left static?

Answer

A static program stops producing gains once the body adapts to the current load, a concept related to progressive overload. Built-in progression criteria let the client safely advance the program between visits without needing constant supervision.

#208 · Care Planning

Scenario: client has low health literacy and struggles to remember verbal exercise instructions. What self-management strategy helps?

Answer

Providing simple written or picture-based handouts, using teach-back to confirm understanding, and keeping the number of exercises small can improve adherence. Video demonstrations on a phone are also a practical option many clients can access at home.

#209 · Care Planning

Term: teach-back method

Answer

Teach-back is a communication technique where the therapist asks the client to explain or demonstrate the instructions in their own words. It confirms understanding and reveals gaps in education before the client leaves the session.

#210 · Care Planning

What factors improve adherence to a home exercise program?

Answer

Adherence improves when the program is simple, matches the client's goals, fits their schedule, and includes clear written instructions. Involving the client in choosing exercises and checking in on barriers at each visit also supports long-term follow-through.

#211 · Care Planning

Scenario: client reports they stopped doing their HEP because it caused increased pain. What should the therapist do?

Answer

The therapist should reassess the exercise, the dose, and the client's pain response rather than simply telling them to push through. The program may need modification in load, range, or frequency, along with reassurance about what pain levels are acceptable.

#212 · Care Planning

Term: pain neuroscience education

Answer

Pain neuroscience education teaches clients how pain is produced by the nervous system and does not always mean tissue damage is occurring. It is used in chronic pain care planning to reduce fear avoidance and support engagement in active rehabilitation.

#213 · Care Planning

What is self-efficacy and why does it matter in care planning?

Answer

Self-efficacy is a person's belief in their own ability to carry out a specific task or manage their condition. Higher self-efficacy is linked to better adherence and outcomes, so building small successes into the plan can strengthen this belief over time.

#214 · Care Planning

Scenario: client with chronic condition wants to know how to tell if a flare-up needs professional help versus self-management. What should the plan teach?

Answer

The plan should give the client clear warning signs, such as new numbness, unrelieved severe pain, or systemic symptoms like fever, that mean they should seek care promptly. It should also outline what milder flare-ups they can manage on their own with rest, modified activity, or previously taught strategies.

#215 · Care Planning

Term: activity pacing

Answer

Activity pacing means breaking tasks into manageable portions with planned rest breaks instead of pushing through until symptoms flare and then crashing. It is commonly taught for chronic pain and fatigue-related conditions to build a steady, sustainable activity level.

#216 · Care Planning

Scenario: client with chronic fatigue tends to overdo activity on good days and crash for several days after. What self-management principle applies?

Answer

This boom-and-bust pattern is addressed by activity pacing, setting a consistent baseline of activity regardless of how good the client feels that day. Gradual, steady increases in the baseline are safer than large swings driven by symptom fluctuation.

#217 · Care Planning

Why should a care plan include goals for family or caregiver involvement in some cases?

Answer

Caregivers often assist with transfers, home exercise supervision, or safety monitoring, especially for clients with cognitive or significant physical limitations. Training the caregiver directly supports carryover of the plan and reduces risk of injury to both parties.

#218 · Care Planning

Term: discharge planning

Answer

Discharge planning is the process of preparing a client to safely transition out of active physiotherapy care, including confirming goals are met, arranging follow-up if needed, and providing a maintenance plan. It should begin early in the episode of care, not only at the final visit.

#219 · Care Planning

What are common criteria used to decide a client is ready for discharge?

Answer

Common criteria include the client meeting their functional goals, reaching a plateau in progress despite continued treatment, or reporting that current symptoms no longer limit their meaningful activities. Safety with independent management of their condition is also required before discharge.

#220 · Care Planning

Scenario: client has plateaued in strength gains over the last 3 sessions despite an appropriately progressed program. What does this suggest for the plan?

Answer

A plateau after appropriate progression may indicate the client is nearing their maximum expected recovery for this episode of care and discharge with a maintenance program should be discussed. It does not automatically mean treatment failed, since some plateau is expected near the end of a recovery curve.

#221 · Care Planning

Term: maintenance program

Answer

A maintenance program is a simplified, sustainable set of exercises given at discharge to help the client keep the gains they achieved during active treatment. It is usually less frequent and less supervised than the active treatment phase.

#222 · Care Planning

Scenario: client is being discharged from outpatient physiotherapy after a total hip replacement and plans to join a community gym. What should the discharge plan include?

Answer

The plan should include a written home or gym program, precautions still relevant at this stage, and criteria for when to seek follow-up care if symptoms return. It is also helpful to communicate the discharge summary to the referring physician or surgeon.

#223 · Care Planning

Term: transition of care

Answer

Transition of care refers to moving a client between levels or settings of service, such as from inpatient rehab to outpatient clinic, or from clinic to community program. Clear communication between providers at this handoff reduces gaps in the client's plan.

#224 · Care Planning

Scenario: client is moving from inpatient stroke rehab to outpatient physiotherapy. What information must be included in the transfer of care documentation?

Answer

The documentation should include current functional status, goals achieved and remaining, precautions such as fall risk or swallowing issues, and equipment currently in use. This ensures the outpatient team can continue the plan without repeating the entire assessment from scratch.

#225 · Care Planning

Term: re-referral criteria

Answer

Re-referral criteria are the specific signs or situations documented at discharge that tell a client when to come back for further physiotherapy care. Clear criteria prevent unnecessary return visits while still catching a genuine setback early.

#226 · Care Planning

Scenario: client achieves all functional goals early, but the therapist believes a few more sessions would still add small gains. What should guide the decision?

Answer

The decision should be guided by the client's own priorities, cost and access considerations, and whether the additional gains are functionally meaningful, not just statistically measurable. Client-centered practice means the client has input into whether continuing is worthwhile to them.

#227 · Care Planning

Term: plan of care modification

Answer

Plan of care modification means adjusting goals, interventions, frequency, or timeline based on reassessment findings such as faster or slower progress than expected. Modification should always be documented with the reasoning behind the change.

#228 · Care Planning

Scenario: client is progressing faster than expected and has already met a 4-week goal at week 2. What should happen to the plan?

Answer

The therapist should reassess and set new, more advanced short-term goals with an updated timeline rather than continuing the same exercises unchanged. Session frequency might also be reduced if the client is ready for more independent management.

#229 · Care Planning

Scenario: client is not progressing as expected after 3 weeks of an appropriately dosed program. What should the therapist consider first?

Answer

The therapist should first reconsider the accuracy of the diagnosis, check for adherence barriers, and screen for psychosocial or medical factors that might be limiting recovery. Only after ruling these out should the exercise program itself be assumed to be the main issue.

#230 · Care Planning

Term: clinical reasoning red flags

Answer

Red flags are signs or symptoms during care that suggest a serious underlying condition needing urgent medical referral, such as unexplained weight loss, night pain, or new neurological deficits. Finding a red flag at any point should prompt the therapist to modify or pause the current plan and refer appropriately.

#231 · Care Planning

Scenario: client with treated low back pain develops new bowel and bladder changes during the episode of care. What should the therapist do?

Answer

This is a red flag for possible cauda equina syndrome and requires urgent medical referral, stopping the regular exercise plan for that visit. The therapist should not continue routine treatment until the client has been medically assessed.

#232 · Care Planning

What is the role of evidence-based practice in choosing between two possible interventions?

Answer

Evidence-based practice combines the best available research evidence, the therapist's clinical expertise, and the client's values and preferences. When two interventions have similar evidence, client preference and practicality often become the deciding factor.

#233 · Care Planning

Scenario: strong research evidence supports an intervention, but the client strongly dislikes it and prefers an alternative with weaker evidence. What should the therapist do?

Answer

The therapist should discuss the evidence honestly with the client and try to find a version of the preferred approach that still meets sound clinical principles. Forcing an intervention against strong client preference usually reduces adherence and outcomes even if the evidence favors it.

#234 · Care Planning

Term: clinical practice guideline

Answer

A clinical practice guideline is a document created from a systematic review of evidence that gives recommendations for managing a specific condition. Guidelines help standardize care planning but still need to be adapted to the individual client's presentation.

#235 · Care Planning

Scenario: client with acute non-specific neck pain wants an x-ray before starting treatment. What does the care plan usually recommend?

Answer

Most guidelines recommend imaging is not needed for acute non-specific neck pain without red flags, since it rarely changes management and can increase unnecessary worry. The therapist should explain this reasoning and proceed with an appropriate conservative treatment trial.

#236 · Care Planning

Term: functional outcome measure

Answer

A functional outcome measure is a standardized tool used to quantify a client's ability to perform meaningful activities, such as the Timed Up and Go test or the Disabilities of the Arm, Shoulder and Hand score. These measures give an objective way to track progress toward functional goals.

#237 · Care Planning

Scenario: elderly client's goal is to safely get up from a chair without using their arms. What outcome measure fits best?

Answer

The 30-second sit to stand test or the Timed Up and Go test are both appropriate because they directly measure the functional task described in the goal. Choosing a measure that matches the goal makes reassessment meaningful and easy to interpret.

#238 · Care Planning

Term: minimal clinically important difference (MCID)

Answer

MCID is the smallest change in an outcome score that a client would consider meaningful, as opposed to a change that is only statistically detectable. Comparing progress against the MCID helps decide whether the treatment plan is producing a result that actually matters to the client.

#239 · Care Planning

Scenario: a client's outcome score improved by 2 points, but the MCID for that tool is 10 points. What does this suggest?

Answer

This small change likely does not represent a meaningful clinical improvement even though it is technically a change in score. The therapist should reconsider the current plan and whether goals or interventions need adjustment.

#240 · Care Planning

Term: functional capacity evaluation

Answer

A functional capacity evaluation is a structured assessment of a client's physical ability to perform specific work-related tasks, such as lifting or carrying. It is often used near discharge planning for clients returning to physically demanding jobs to guide safe return-to-work timing.

#241 · Care Planning

Scenario: client's job requires repeated lifting of 25 kg boxes and current tested tolerance is 15 kg. What should the plan target?

Answer

The plan should include progressive strengthening and task simulation exercises aimed at closing that gap, along with reassessing lifting tolerance regularly. Return to full duties should be timed once tolerance safely meets or exceeds the job demand.

#242 · Care Planning

Term: graded return to activity or sport

Answer

Graded return to activity is a staged progression that gradually increases the intensity, duration, and complexity of a specific activity, sport, or work task. Each stage typically has criteria that must be met before advancing, reducing the risk of re-injury.

#243 · Care Planning

Scenario: client recovering from a hamstring strain wants to return to running. What criteria should guide progression before full sprinting?

Answer

Criteria should include pain-free walking, full pain-free stretch and strength testing comparable to the uninjured side, and successful completion of lower intensity jogging before sprinting is added. Progressing by feel alone, without objective criteria, increases re-injury risk.

#244 · Care Planning

Term: interdisciplinary care planning

Answer

Interdisciplinary care planning involves multiple health professionals, such as physiotherapy, occupational therapy, and nursing, working together with shared goals for the client. Good communication between team members prevents conflicting instructions and supports a consistent plan.

#245 · Care Planning

Scenario: physiotherapist and occupational therapist have different goals for the same client's home safety after discharge. What should happen?

Answer

The team should hold a case conference or communicate directly to align on shared, non-conflicting goals before the client goes home. Conflicting instructions from different team members can confuse the client and undermine the whole plan.

#246 · Care Planning

Term: informed consent in treatment planning

Answer

Informed consent means the client understands the proposed intervention, its expected benefits, risks, and reasonable alternatives, and agrees to proceed. Consent should be revisited whenever the plan changes significantly, not just obtained once at the first visit.

#247 · Care Planning

Scenario: therapist wants to add a new manual therapy technique not previously discussed with the client. What must happen first?

Answer

The therapist must explain the new technique, its purpose, and any risks, and obtain the client's consent before performing it. Skipping this step even for a minor addition to the plan is a professional and ethical issue.

#248 · Care Planning

Term: goal attainment scaling

Answer

Goal attainment scaling is a method of individualizing outcome measurement by defining a range of possible results, from much worse than expected to much better than expected, around a specific goal. It allows progress tracking even when a standard outcome measure does not fit the client's unique goal.

#249 · Care Planning

Scenario: client has a rare condition without a standard outcome measure, but the therapist still needs to track progress toward a personal goal. What tool fits?

Answer

Goal attainment scaling fits well here because it is built around the client's own individualized goal rather than requiring a standardized instrument. The therapist defines what better and worse than expected outcomes would look like ahead of time.

#250 · Care Planning

Term: fall risk screening in discharge planning

Answer

Fall risk screening uses tools such as the Berg Balance Scale or Timed Up and Go to identify clients at higher risk of falling before they are discharged home. A positive screen should trigger environmental modification recommendations, an assistive device assessment, or referral to a falls prevention program.

#251 · Care Planning

Scenario: elderly client scores in the high fall risk range at discharge from inpatient rehab. What should the discharge plan include?

Answer

The plan should include a home safety assessment or referral, an appropriate assistive device, caregiver education, and a clear follow-up pathway if further falls occur. Discharge should not proceed without addressing this identified risk.

#252 · Care Planning

Term: assistive device prescription in care planning

Answer

Assistive device prescription involves matching the right device, such as a cane, walker, or wheelchair, to the client's current strength, balance, and functional needs. The device choice should be reassessed as the client's status changes throughout the episode of care.

#253 · Care Planning

Scenario: client initially needed a walker after hip fracture surgery but has now regained good single leg balance and strength. What should the plan do?

Answer

The plan should reassess and progress the client toward a cane or no device if safe, since continuing an unnecessary device can limit further functional gains. Device downgrade should always be based on objective balance and strength testing, not just time elapsed.

#254 · Care Planning

Term: environmental modification

Answer

Environmental modification means changing the physical home or work environment, such as adding grab bars or removing throw rugs, to reduce risk and support independence. It is often part of discharge planning for clients with mobility or balance limitations.

#255 · Care Planning

Scenario: client lives alone with stairs to the only bathroom and has reduced lower limb strength after a prolonged hospital stay. What should the discharge plan consider?

Answer

The plan should assess the client's ability to safely manage stairs, consider a temporary commode or bedside solution if needed, and possibly involve occupational therapy for a home safety visit. Safe stair management should be objectively tested before allowing unsupervised discharge home.

#256 · Care Planning

Term: red flag versus yellow flag

Answer

A red flag suggests possible serious pathology needing urgent medical referral, such as cauda equina symptoms. A yellow flag is a psychosocial risk factor, such as fear avoidance or job dissatisfaction, that predicts slower recovery and should shape the rehabilitation approach rather than trigger urgent referral.

#257 · Care Planning

Scenario: client with chronic shoulder pain shows strong fear of reinjury and reports low mood about their recovery. What flag does this represent and what should the plan include?

Answer

This represents a yellow flag, and the plan should include education to address the fear, graded exposure to feared movements, and consideration of screening for mood concerns if significant. Ignoring this psychosocial factor and focusing only on physical exercise often leads to poor outcomes.

#258 · Care Planning

Term: dosage parameters for aerobic exercise prescription

Answer

Aerobic exercise dosage is typically described using frequency, intensity, time, and type, often called the FITT principle. Each parameter should be individually set based on the client's condition, fitness level, and goals rather than using one generic prescription for everyone.

#259 · Care Planning

Scenario: sedentary client with cardiovascular risk factors is starting an exercise program for the first time. What intensity level is generally recommended to start?

Answer

A moderate intensity level, often described as being able to talk but not sing during activity, is a common safe starting point for a deconditioned client. Intensity can be progressed gradually as tolerance and fitness improve.

#260 · Care Planning

Term: warm-up and cool-down in an exercise plan

Answer

A warm-up gradually raises heart rate and prepares muscles and joints before the main exercise session, while a cool-down gradually lowers heart rate afterward. Both are especially important in cardiac and older adult populations to reduce the risk of adverse cardiovascular events.

#261 · Care Planning

Scenario: client with cardiac history skips the cool-down and stops exercise abruptly. What risk does this create?

Answer

Abruptly stopping exercise can cause blood to pool in the legs and a sudden drop in blood pressure, increasing the risk of dizziness or fainting. The plan should reinforce a proper gradual cool-down for cardiac clients every session.

#262 · Care Planning

Term: contraindication versus precaution in exercise prescription

Answer

A contraindication means the exercise or activity should not be performed at all because of significant risk. A precaution means the activity can be done but requires extra caution, modification, or monitoring.

#263 · Care Planning

Scenario: client has an unhealed surgical incision and the plan calls for resistance band exercises directly across that area. What should the therapist do?

Answer

The therapist should modify the exercise to avoid direct tension across the unhealed incision as a precaution, or delay that specific exercise until adequate healing has occurred. The rest of the exercise plan can usually continue with appropriate modification.

#264 · Care Planning

Term: functional reach test

Answer

The functional reach test measures how far a person can reach forward without losing balance or taking a step, used as a quick balance screening tool. It is often included in a plan targeting fall risk in older adults or neurological populations.

#265 · Care Planning

Scenario: client with Parkinson disease has a goal of safely reaching into a high cupboard without losing balance. What test fits well for tracking this goal?

Answer

The functional reach test fits this goal well because it directly measures forward reaching balance in a way similar to the functional task described. Reassessing with the same test lets the therapist show objective progress toward that specific goal.

#266 · Care Planning

Term: education as an intervention category

Answer

Education is considered an active intervention in its own right, covering topics like condition understanding, activity modification, and self-management strategies. It should be planned and documented with the same intentionality as an exercise prescription, not treated as an afterthought.

#267 · Care Planning

Scenario: client with newly diagnosed osteoarthritis asks whether running will damage their joints further. What should the plan include?

Answer

The plan should include education explaining that current evidence does not support the idea that moderate running causes or worsens osteoarthritis in most people, and that appropriate activity is generally beneficial. This kind of accurate education can reduce unnecessary fear and support continued activity.

#268 · Care Planning

Term: chronic disease self-management program

Answer

A chronic disease self-management program is a structured education and skill-building course, often group based, that teaches clients strategies for managing symptoms, medication, exercise, and communication with health providers. Physiotherapists may refer clients to or run components of these programs as part of long-term care planning.

#269 · Care Planning

Scenario: client with well-managed type 2 diabetes and peripheral neuropathy is starting a walking program for weight management. What safety point should the plan include?

Answer

The plan should include daily foot inspection and appropriate footwear because reduced sensation from neuropathy increases the risk of unnoticed skin breakdown from new activity. Blood glucose monitoring around exercise sessions may also need to be reviewed with the client's care team.

#270 · Care Planning

Term: shared decision making

Answer

Shared decision making is a collaborative process where the therapist presents evidence-based options and the client shares their values and preferences, and together they choose the plan. This differs from the therapist simply telling the client what to do without discussion.

#271 · Care Planning

Scenario: two treatment options exist for a client's condition with similar evidence but different time commitments. What approach should the therapist use to decide?

Answer

The therapist should use shared decision making, presenting both options clearly along with pros and cons, and let the client's schedule and preference help guide the final choice. This respects client autonomy while still applying clinical expertise.

#272 · Care Planning

Term: functional independence measure (FIM)

Answer

The FIM is a standardized scale that scores a client's level of independence across activities like self-care, mobility, and cognition, commonly used in inpatient rehabilitation settings. It helps set realistic discharge goals and can predict the amount of assistance needed after leaving the facility.

#273 · Care Planning

Scenario: inpatient rehab client's FIM mobility score shows they need moderate assistance for transfers. What does the discharge plan need to address?

Answer

The plan needs to address whether a caregiver will be available to provide that level of assistance at home, or whether further inpatient time or additional equipment is needed before safe discharge. Discharge should not proceed until the assistance gap between current status and home support is resolved.

#274 · Care Planning

Term: dosage progression for gait training

Answer

Gait training progression typically moves from parallel bars or a supportive device on level ground, to unsupported walking, to varied terrain, stairs, and community distances. Progression criteria should include safety, speed, and endurance, not just distance walked.

#275 · Care Planning

Scenario: client can walk 50 meters safely indoors with a rollator but the discharge goal is community ambulation to a nearby store 200 meters away. What should the plan add?

Answer

The plan should add progressive distance and endurance training along with practice on outdoor surfaces such as curbs and uneven pavement before discharge. Testing in a setting similar to the real community environment gives a more accurate picture of readiness.

#276 · Care Planning

Term: precautions after total hip replacement (posterior approach)

Answer

Common precautions include avoiding hip flexion past 90 degrees, avoiding hip internal rotation, and avoiding crossing the legs, though exact precautions vary by surgeon and approach. The care plan must reflect the specific surgeon's protocol rather than a generic assumption.

#277 · Care Planning

Scenario: client with a posterior approach hip replacement wants to resume yoga with deep hip flexion poses at 3 weeks post-op. What should the plan address?

Answer

The plan should educate the client to avoid deep hip flexion and internal rotation poses until precautions are lifted by the surgeon, typically around 6 to 12 weeks. Alternative modified positions can be suggested to keep the client engaged in activity safely.

#278 · Care Planning

Term: deconditioning

Answer

Deconditioning refers to a decline in cardiovascular fitness, strength, and functional capacity that occurs after a period of inactivity or illness. Care plans for hospitalized or post-surgical clients often need to address deconditioning in addition to the primary diagnosis.

#279 · Care Planning

Scenario: client recovering from a prolonged ICU stay for pneumonia now has generalized weakness unrelated to the lungs directly. What should the plan focus on?

Answer

The plan should focus on general reconditioning, including progressive mobility, strengthening, and endurance training, in addition to any remaining respiratory needs. This generalized weakness after critical illness is sometimes called ICU-acquired weakness and needs its own targeted rehabilitation.

#280 · Care Planning

Term: energy conservation techniques

Answer

Energy conservation techniques include planning activities in advance, sitting instead of standing for tasks when possible, and spacing out demanding activities across the day. They are often taught to clients with cardiorespiratory or fatigue-related conditions to help them accomplish daily tasks without excessive symptom flare.

#281 · Care Planning

Scenario: client with severe COPD becomes very breathless doing simple showering tasks. What should the self-management plan teach?

Answer

The plan should teach energy conservation strategies such as using a shower chair, sitting while dressing, and pacing the task with breathing control techniques like pursed lip breathing. Reducing unnecessary standing and multitasking during self-care lowers the oxygen demand of the activity.

#282 · Care Planning

Term: reassessment interval

Answer

The reassessment interval is the planned point at which the therapist formally re-measures progress against baseline and goals, rather than relying only on subjective impressions each visit. It should be built into the plan from the start, not added only when things seem to be going poorly.

#283 · Care Planning

Scenario: therapist has been seeing a client weekly for 8 weeks without a single formal reassessment of the original outcome measures. What is the concern?

Answer

Without periodic reassessment there is no objective evidence the plan is working, which is both a clinical and documentation problem. The therapist should build in regular reassessment points to justify continued treatment and adjust the plan as needed.

#284 · Care Planning

Term: functional goal hierarchy

Answer

A functional goal hierarchy organizes goals from simple foundational tasks toward more complex functional activities, such as progressing from sitting balance, to standing balance, to walking. Building the plan in this order helps ensure each stage supports the next.

#285 · Care Planning

Scenario: client with a recent stroke cannot yet maintain unsupported sitting balance, but the family is asking about a walking goal. How should the therapist respond?

Answer

The therapist should explain that sitting balance is a foundational goal that typically must be achieved before standing and walking goals become realistic. The plan should target sitting balance first while keeping the family informed about the expected progression toward walking.

#286 · Care Planning

Term: dual task training

Answer

Dual task training involves performing a motor task, such as walking, while simultaneously completing a cognitive or secondary task, such as counting backward or carrying an object. It is used to prepare clients for real-world situations where attention must be divided, and is common in fall prevention and neuro rehab plans.

#287 · Care Planning

Scenario: older client walks safely alone but has fallen in the past while talking on the phone and walking at the same time. What training should be added?

Answer

Dual task training combining walking with a conversation or cognitive task should be added to better match the real situation that caused the previous fall. Practicing only single task walking would not adequately prepare the client for this specific fall risk scenario.

#288 · Care Planning

Term: work conditioning versus work hardening

Answer

Work conditioning focuses on improving general physical capacity, such as strength and endurance, related to job demands. Work hardening is a more comprehensive, often interdisciplinary program that also simulates actual job tasks and addresses behavioral and psychosocial return-to-work barriers.

#289 · Care Planning

Scenario: client has good physical recovery from a back injury but is highly anxious about re-injury and reluctant to return to their physical job. What program level fits best?

Answer

Work hardening fits better here because it addresses the psychosocial and behavioral barriers alongside the physical job simulation, not just the physical capacity component. Work conditioning alone would not target the anxiety driving the reluctance to return.

#290 · Care Planning

Term: documentation of clinical reasoning in a care plan

Answer

Documentation should clearly show why specific goals and interventions were chosen, linking assessment findings to the plan rather than just listing exercises. This supports continuity of care if another provider takes over and justifies treatment decisions for funding or legal review.

#291 · Care Planning

Scenario: a colleague reviewing a chart cannot tell why a particular exercise was chosen for a client's condition. What is missing from the documentation?

Answer

The clinical reasoning link between the assessment findings and the chosen intervention is missing from the notes. Good documentation should briefly explain the rationale, not just record what was done.

#292 · Care Planning

Term: client-reported outcome measure (PROM)

Answer

A patient-reported outcome measure is a questionnaire completed directly by the client describing their own symptoms, function, or quality of life, such as the Neck Disability Index. These measures capture the client's perspective, which may differ from what the therapist observes in objective testing.

#293 · Care Planning

Scenario: client's objective range of motion has fully normalized, but their patient-reported outcome score still shows high disability. What should the plan consider?

Answer

The plan should consider that psychosocial factors, fear, or other functional barriers may still be limiting the client even though the physical impairment has resolved. Relying only on objective measures here would miss an important part of the picture.

#294 · Care Planning

Term: cardiac rehab exercise contraindication

Answer

Common contraindications to starting or continuing a cardiac rehab exercise session include unstable angina, uncontrolled arrhythmia, resting systolic blood pressure above a set threshold, or significant unresolved symptoms like chest pain. The therapist should always check current vital signs and symptoms before starting each session.

#295 · Care Planning

Scenario: client in cardiac rehab reports new chest pain and reports it just before their scheduled exercise session. What should happen to the plan for that day?

Answer

Exercise should be stopped or not started, and the client should be assessed by medical staff before any physiotherapy exercise resumes. Continuing the planned exercise session in this situation would be unsafe regardless of how the plan was originally written.

#296 · Care Planning

Term: return to sport criteria after ACL reconstruction

Answer

Return to sport criteria typically include meeting strength symmetry benchmarks between limbs, passing hop testing, and demonstrating good movement quality on functional tests, often around 9 months or later post-surgery. Time since surgery alone is not considered sufficient evidence of readiness.

#297 · Care Planning

Scenario: client is 6 months post ACL reconstruction and wants to return to competitive soccer, but strength testing shows a 25 percent deficit on the surgical side. What should the plan recommend?

Answer

The plan should recommend continuing strengthening and delaying return to full competitive play until strength symmetry and functional testing benchmarks are met. Returning at this strength deficit carries a meaningfully higher risk of re-injury.

#298 · Care Planning

Term: goal modification due to a new medical diagnosis

Answer

If a client develops a new medical condition during their episode of care, such as a new cardiac diagnosis, the existing goals and plan must be reassessed for continued safety and appropriateness. The therapist may need to consult with the medical team before continuing the original plan unchanged.

#299 · Care Planning

Scenario: client midway through a knee rehab program is newly diagnosed with atrial fibrillation by their physician. What should the therapist do with the current plan?

Answer

The therapist should review the plan for any needed precautions related to the new cardiac diagnosis, possibly consulting the physician about exercise clearance, before continuing unchanged. Safety screening should be updated any time a significant new diagnosis appears during care.

#300 · Care Planning

Term: comprehensive discharge summary

Answer

A comprehensive discharge summary documents the client's status at admission, interventions provided, outcome measure changes, goals met or not met, and any recommendations for ongoing self-management or follow-up. It serves as the key communication tool between the discharging therapist and any future care providers.

#301 · Client Safety and Client-Centred Care

What are the four required elements of valid informed consent?

Answer

The client must be given information about the proposed treatment, understand it, receive it voluntarily without coercion, and have the capacity to consent. If any one of these is missing, the consent is not valid. Consent can be withdrawn by the client at any point, even mid-treatment.

#302 · Client Safety and Client-Centred Care

Implied consent vs express consent

Answer

Implied consent is shown through a client's actions, such as lying down on the plinth when asked. Express consent is given verbally or in writing and is required for higher-risk procedures like internal pelvic floor work or joint manipulation. Both types must still meet the standard of being informed.

#303 · Client Safety and Client-Centred Care

A client with dementia is scheduled for treatment. How should a physiotherapist check capacity?

Answer

Capacity is assessed task by task, not as an all-or-nothing label. The therapist checks whether the client can understand the specific treatment information, weigh the risks and benefits, and communicate a choice for that particular decision. A dementia diagnosis alone does not mean the client lacks capacity.

#304 · Client Safety and Client-Centred Care

Substitute decision maker

Answer

A substitute decision maker consents to treatment on behalf of a client who has been assessed as lacking capacity for that decision. The therapist should still involve the client as much as possible and explain the treatment in a way the client can follow. Provincial legislation sets the hierarchy of who qualifies as a substitute decision maker.

#305 · Client Safety and Client-Centred Care

Absolute contraindication to therapeutic exercise

Answer

Unstable angina and acute myocardial infarction within the last two days are absolute contraindications to exercise testing and training. Uncontrolled cardiac arrhythmia causing symptoms and acute aortic dissection are also absolute contraindications. Exercise must be withheld until the client is medically cleared.

#306 · Client Safety and Client-Centred Care

Relative contraindication to therapeutic exercise

Answer

Uncontrolled hypertension, moderate valvular heart disease, and electrolyte abnormalities are relative contraindications. Exercise may still proceed but only after weighing the benefit against the risk and often after physician consultation. Close monitoring during the session becomes essential in these cases.

#307 · Client Safety and Client-Centred Care

Name three ACSM indications to stop an exercise session immediately

Answer

Onset of angina or angina-like chest pain, a drop in systolic blood pressure with increasing workload, and signs of poor perfusion such as pallor, cyanosis, or cold clammy skin all mean the session must stop right away. Sustained ventricular tachycardia and the client's request to stop are also immediate stop signs. The client should be monitored and a physician contacted if symptoms persist.

#308 · Client Safety and Client-Centred Care

Contraindication to spinal manipulation (HVLA)

Answer

Cauda equina syndrome, vertebral fracture, spinal malignancy, and significant osteoporosis are contraindications to high velocity low amplitude manipulation. Vertebral artery insufficiency and inflammatory arthropathies affecting the upper cervical spine, such as rheumatoid arthritis with atlantoaxial instability, also rule out cervical manipulation. A thorough screen for these red flags must happen before any manipulative technique.

#309 · Client Safety and Client-Centred Care

The 5 Ds and 3 Ns of vertebral artery insufficiency

Answer

The 5 Ds are dizziness, diplopia, dysarthria, dysphagia, and drop attacks. The 3 Ns are nausea, nystagmus, and numbness on one side of the face. Any of these signs during a cervical spine assessment means manipulation and sustained end range positioning should be avoided and the client referred for medical assessment.

#310 · Client Safety and Client-Centred Care

A client reports saddle anesthesia, bladder retention, and bilateral leg weakness during a low back assessment. What is this and what should the therapist do?

Answer

These are classic signs of cauda equina syndrome, a surgical emergency caused by compression of the lower spinal nerve roots. The client needs immediate referral to an emergency department, not routine physiotherapy treatment. Delayed treatment can cause permanent bowel, bladder, and lower limb dysfunction.

#311 · Client Safety and Client-Centred Care

Autonomic dysreflexia

Answer

Autonomic dysreflexia is a medical emergency seen in clients with spinal cord injury at or above T6, triggered by a noxious stimulus below the level of injury such as a full bladder or a tight strap. Signs include a sudden spike in blood pressure, pounding headache, flushing above the injury level, and sweating. The therapist should sit the client upright, remove the trigger, loosen tight clothing, and call for emergency help if symptoms do not resolve quickly.

#312 · Client Safety and Client-Centred Care

Signs of acute compartment syndrome

Answer

The classic signs are pain out of proportion to the injury, pain with passive stretch of the muscles in that compartment, and a tense swollen limb. Pallor, paresthesia, and pulselessness are late signs and mean permanent damage may already be happening. This is an orthopedic emergency requiring immediate transfer, not physiotherapy treatment.

#313 · Client Safety and Client-Centred Care

Why is Homans sign considered unreliable for detecting DVT?

Answer

Homans sign, calf pain with passive ankle dorsiflexion, has poor sensitivity and specificity and can be positive or negative regardless of whether a deep vein thrombosis is present. Relying on it alone can miss a clot or falsely alarm the therapist. Clinical prediction tools like the Wells score combined with clinical judgment and physician referral are more appropriate.

#314 · Client Safety and Client-Centred Care

A client suddenly develops facial droop, arm weakness, and slurred speech during a session. What should the therapist do?

Answer

This presentation matches the FAST warning signs of stroke: face drooping, arm weakness, speech difficulty, and time to call emergency services. The therapist should stop treatment immediately and call emergency medical services rather than waiting to see if symptoms pass. Noting the exact time symptoms started is important information for the emergency team.

#315 · Client Safety and Client-Centred Care

Anaphylaxis during treatment

Answer

Signs include hives, swelling of the face or throat, difficulty breathing, and a sudden drop in blood pressure. The therapist should call emergency services immediately, help administer the client's own epinephrine auto-injector if available, and monitor airway and vital signs until paramedics arrive. Treatment sessions should never continue once anaphylaxis is suspected.

#316 · Client Safety and Client-Centred Care

Contraindication to superficial heat therapy

Answer

Superficial heat should be avoided over areas of acute inflammation, impaired sensation, impaired circulation, malignancy, and open wounds. It is also avoided directly over the pregnant abdomen and over areas with recent hemorrhage. Skin should be checked before and after application for any adverse reaction.

#317 • Client Safety and Client-Centred Care

Contraindication to cryotherapy (cold application)

Answer

Cold should not be applied over areas with impaired circulation, Raynaud's phenomenon, cryoglobulinemia, or cold urticaria, since it can trigger a harmful vascular or allergic response. Impaired sensation and open wounds are also contraindications. Clients with these conditions can have significant tissue damage from what looks like a routine ice pack.

#318 • Client Safety and Client-Centred Care

Contraindication to therapeutic ultrasound

Answer

Ultrasound should not be used over the pregnant uterus, malignant tissue, active epiphyseal growth plates in children, or an area with a pacemaker or other implanted electronic device. It is also avoided over the eyes, over acute infection, and over areas of thrombophlebitis. Sensation should be intact in the treatment area before applying ultrasound.

#319 · Client Safety and Client-Centred Care

Contraindication to electrical stimulation

Answer

Electrical stimulation is contraindicated over the carotid sinus, over the chest in a client with a cardiac pacemaker or implanted defibrillator, and over the pregnant abdomen. It should not be applied over areas of active cancer or through the head. Broken skin and metal implants at the surface also require caution.

#320 · Client Safety and Client-Centred Care

Why does an implanted pacemaker change modality selection?

Answer

Electrical currents from TENS, neuromuscular stimulation, or diathermy can interfere with the pacemaker's sensing circuit and disrupt its programmed rhythm. Many devices also have manufacturer warnings against use near MRI and diathermy units. The therapist should check with the client's cardiology team before using any electrical modality.

#321 · Client Safety and Client-Centred Care

A diabetic client with peripheral neuropathy needs a hot pack. What should the therapist check first?

Answer

Sensation in the treatment area should be tested before applying any heat modality, since impaired sensation prevents the client from feeling and reporting a burn. Skin should also be inspected for any existing ulcers or breakdown. If sensation is significantly reduced, heat should be avoided or applied at a lower intensity with frequent skin checks.

#322 · Client Safety and Client-Centred Care

Osteoporosis and manual therapy

Answer

Significant osteoporosis is a contraindication to high velocity thrust techniques and forceful joint mobilizations because of increased fracture risk. Gentler grades of mobilization and exercise-based approaches are safer choices. Bone density history and fracture risk should be screened before any manual technique is chosen.

#323 · Client Safety and Client-Centred Care

Why is exertional hypotension a red flag during an exercise session?

Answer

A drop in systolic blood pressure of 10 mmHg or more as workload increases suggests the heart cannot keep up with demand, which can signal serious cardiac pathology such as left ventricular dysfunction. This finding is an indication to stop the test or exercise session right away. The client should be seated or positioned safely and referred for medical follow up.

#324 · Client Safety and Client-Centred Care

Borg Rating of Perceived Exertion scale

Answer

The Borg scale, typically 6 to 20, lets a client self-report how hard they feel they are working during exercise. It is used alongside heart rate and blood pressure to monitor safety and tolerance, especially in clients on medications like beta blockers that blunt the heart rate response. A rating that climbs faster than expected for the workload can flag a problem before objective vitals change.

#325 · Client Safety and Client-Centred Care

What vital signs should be monitored during a cardiac rehab exercise session?

Answer

Heart rate, blood pressure, oxygen saturation, and perceived exertion are typically monitored before, during, and after exercise. The client's breathing pattern, skin color, and any chest pain or dizziness should also be observed continuously. Abnormal trends in any of these are reasons to slow down or stop the session.

#326 · Client Safety and Client-Centred Care

Standard precautions

Answer

Standard precautions apply to every client interaction regardless of known infection status and include hand hygiene, use of personal protective equipment when contact with body fluids is possible, safe handling of sharps, and cleaning of shared equipment between clients. They assume that any client could carry an infectious agent. This baseline level of precaution never gets skipped, even for low-risk looking clients.

#327 · Client Safety and Client-Centred Care

Transmission-based precautions: contact, droplet, and airborne

Answer

Contact precautions use gown and gloves for organisms spread by touch, such as *C. difficile* or MRSA. Droplet precautions add a surgical mask for pathogens spread by large respiratory droplets, such as influenza. Airborne precautions require an N95 respirator and a negative pressure room for pathogens that travel in small particles, such as tuberculosis or measles.

#328 · Client Safety and Client-Centred Care

Why is alcohol-based hand sanitizer not enough for a client with *C. difficile*?

Answer

Clostridioides difficile produces spores that alcohol-based sanitizers do not reliably kill. Hands must be washed with soap and running water, which physically removes the spores. Contact precautions with gown and gloves are also required when entering the room.

#329 · Client Safety and Client-Centred Care

The four moments of hand hygiene

Answer

Hands should be cleaned before touching a client, before a clean or aseptic procedure, after a body fluid exposure risk, and after touching a client or their immediate surroundings. This framework applies in clinics, hospitals, and home visits alike. Missing any one of these moments creates a chance to spread infection between clients.

#330 · Client Safety and Client-Centred Care

Correct order for removing personal protective equipment (doffing)

Answer

Gloves are removed first since they are the most contaminated item, followed by hand hygiene, then the gown, then eye protection, and finally the mask or respirator with hand hygiene again at the end. Removing items in the wrong order risks transferring contamination to skin or clothing. Facilities post the exact sequence and it should be followed every time, not from memory alone.

#331 · Client Safety and Client-Centred Care

Needlestick injury protocol

Answer

The area should be washed immediately with soap and water, the injury reported to a supervisor right away, and the incident documented following workplace policy. Occupational health should be contacted promptly since post-exposure prophylaxis for bloodborne pathogens is time sensitive. The source client's status should be assessed through appropriate channels, not by the injured therapist alone.

#332 · Client Safety and Client-Centred Care

Near miss vs adverse event

Answer

A near miss is an event that could have caused harm but did not, such as almost giving a modality to the wrong client before catching the error. An adverse event is an incident that did cause unintended harm to the client. Both should be reported and documented since near misses reveal system weaknesses before an actual injury occurs.

#333 · Client Safety and Client-Centred Care

Why report near misses even when no one was harmed?

Answer

Reporting near misses lets a clinic identify patterns and fix unsafe processes before they cause real harm. It supports a culture of safety rather than blame, encouraging staff to speak up. Regulatory bodies and quality improvement programs rely on this data to prevent repeat incidents.

#334 · Client Safety and Client-Centred Care

A client falls off the plinth during treatment. What is the therapist's first responsibility?

Answer

The therapist should first check the client for injury and provide any needed first aid or call for emergency help. Once the client is stabilized, the incident must be documented factually in the chart and reported through the clinic's adverse event process. The client and, if appropriate, their family should be told what happened in a transparent way.

#335 · Client Safety and Client-Centred Care

Documentation after an adverse event

Answer

Documentation should include an objective factual account of what happened, the client's response, actions taken, and who was notified, without speculation or blame. Regulatory colleges generally require this to be recorded promptly, close to the time of the event. The incident report itself is often kept separate from the clinical chart per organizational policy.

#336 · Client Safety and Client-Centred Care

Client-centred care, defined

Answer

Client-centred care means treatment decisions are built around the individual client's goals, values, and preferences rather than a fixed protocol. The client is treated as an active partner in setting goals and choosing between treatment options. This approach improves adherence and respects the client's right to self-determination.

#337 · Client Safety and Client-Centred Care

A client wants to stop a treatment technique partway through because of discomfort. What must the therapist do?

Answer

The therapist must stop immediately since consent can be withdrawn at any time, even mid-treatment, and continuing without consent could be considered assault. The reason for stopping should be discussed and an alternative approach explored together. This respects client autonomy even if the therapist believes the technique was appropriate.

#338 · Client Safety and Client-Centred Care

Cultural safety vs cultural competence

Answer

Cultural competence focuses on the provider gaining knowledge and skills about other cultures. Cultural safety goes further and centers on the client's own experience, asking whether the client feels respected and free from discrimination during care, as defined by the client rather than the provider. In Canada this concept is particularly emphasized in care for Indigenous clients given the history of colonization in health care.

#339 · Client Safety and Client-Centred Care

Why use a professional interpreter instead of a client's family member?

Answer

A professional interpreter provides accurate, unbiased translation and maintains confidentiality, while a family member may filter, add opinions, or feel uncomfortable relaying sensitive health information. Using family members can also compromise the client's privacy and true informed consent. Accessible interpretation services support both safety and client-centred care.

#340 · Client Safety and Client-Centred Care

Accessibility advocacy in a physiotherapy clinic

Answer

This includes making sure treatment areas, plinths, and washrooms are usable by clients with mobility devices, offering materials in plain language or alternate formats, and scheduling flexibility for clients with transportation barriers. Advocacy also means raising these barriers with clinic management when they are identified, not just working around them silently. The goal is that a client's disability should not determine the quality of care received.

#341 · Client Safety and Client-Centred Care

A client using a wheelchair cannot access the second-floor clinic because the elevator is broken. What should the therapist do?

Answer

The therapist should offer to reschedule at an accessible location, arrange a ground floor room if one is available, or discuss a virtual visit if appropriate for that treatment. The barrier should also be reported to management so it gets fixed for future appointments. Simply canceling without offering alternatives fails the duty of accessible care.

#342 · Client Safety and Client-Centred Care

Duty to report suspected elder abuse

Answer

Physiotherapists who observe signs of physical, emotional, or financial abuse, or neglect of an older client, generally have a professional obligation to report to the appropriate authority, and in several provinces this is a legislated duty for clients in care facilities. The therapist should document objective findings such as bruising patterns or signs of neglect. Confidentiality does not override this duty to protect a vulnerable client from harm.

#343 · Client Safety and Client-Centred Care

Duty to report suspected child abuse

Answer

In every Canadian province and territory, anyone who suspects a child is being abused or neglected must report it to child protection services, and this duty overrides confidentiality. The therapist does not need proof, only reasonable grounds for suspicion. Failing to report can itself result in legal and professional consequences for the therapist.

#344 · Client Safety and Client-Centred Care

Confidentiality and its exceptions

Answer

Client health information is confidential and should only be shared with consent or on a need to know basis with the care team. Exceptions include mandatory reporting laws such as child or vulnerable adult abuse, a court order, or situations involving a genuine risk of serious harm to the client or another identifiable person. Outside of these exceptions, information cannot be shared with employers, family, or insurers without the client's permission.

#345 · Client Safety and Client-Centred Care

Duty to warn

Answer

If a client discloses a specific, credible, and imminent threat to harm an identifiable person, the therapist may be permitted or required to break confidentiality to warn that person or notify authorities. This exception is narrow and requires a serious and specific risk, not a vague statement. Clinics should have a clear policy and the therapist should consult a supervisor or regulatory body when this situation arises.

#346 · Client Safety and Client-Centred Care

Advance directive

Answer

An advance directive is a document in which a client states their wishes for future health care in case they lose capacity to make decisions later. Physiotherapists should be aware such a document may exist and respect its instructions when relevant to treatment decisions. It works alongside, not instead of, ongoing conversations about the client's current wishes whenever possible.

#347 · Client Safety and Client-Centred Care

A client with a healing surgical incision wants to start hydrotherapy. What should the therapist check first?

Answer

The incision should be assessed to confirm it is fully closed and free of signs of infection such as redness, discharge, or increased warmth before submerging it in water. Physician clearance is typically needed given infection risk with open or recently healed wounds. Pool water quality and chlorine levels should also meet clinic safety standards.

#348 · Client Safety and Client-Centred Care

Falls risk in an older adult client

Answer

Key risk factors include a history of previous falls, gait and balance impairment, polypharmacy, postural hypotension, and impaired vision. A validated screening tool such as the Timed Up and Go test helps quantify risk objectively. Identified risk should prompt both an exercise plan and environmental or footwear recommendations to reduce future falls.

#349 · Client Safety and Client-Centred Care

Restraint use and consent

Answer

Physical or chemical restraint of a client requires clear justification, the least restrictive option available, and generally consent from the client or their substitute decision maker except in true emergencies. Restraint used purely for staff convenience is not acceptable practice. Any use of restraint must be documented with the reason, duration, and monitoring performed.

#350 · Client Safety and Client-Centred Care

Why should a physiotherapist screen for red flags before starting a treatment plan for low back pain?

Answer

Red flag screening rules out serious underlying pathology such as fracture, cauda equina syndrome, infection, or malignancy that mimics mechanical back pain but needs urgent medical management instead of physiotherapy. Missing a red flag can delay life or function threatening diagnoses. This screen should happen at initial assessment and be repeated if new symptoms appear during care.

#351 · Client Safety and Client-Centred Care

Red flags suggesting spinal infection

Answer

Fever, unexplained weight loss, recent bacterial infection elsewhere in the body, night pain that is unrelieved by rest, and a history of IV drug use or immunosuppression raise suspicion of spinal infection. These findings warrant urgent medical referral rather than continued conservative treatment. Blood work and imaging are needed to confirm or rule out the diagnosis.

#352 · Client Safety and Client-Centred Care

Red flags suggesting spinal malignancy

Answer

Unexplained weight loss, age over 50, a personal history of cancer, pain that is constant and worse at night or unrelieved by rest or position change, and failure to improve with conservative treatment are red flags for possible spinal malignancy. These findings should prompt referral back to the physician for further workup. Physiotherapy should not proceed as if the presentation were simple mechanical pain.

#353 · Client Safety and Client-Centred Care

Signs of a deep vein thrombosis in the calf

Answer

Unilateral calf swelling, warmth, redness, and tenderness along the deep veins are classic signs, sometimes accompanied by pitting edema. A DVT is a medical emergency because a piece of the clot can break off and travel to the lungs as a pulmonary embolism. Suspected DVT requires urgent medical referral and vigorous massage or exercise of the limb should be avoided until it is ruled out.

#354 · Client Safety and Client-Centred Care

Signs of pulmonary embolism during a treatment session

Answer

Sudden onset shortness of breath, sharp chest pain that worsens with breathing, rapid heart rate, and coughing up blood are warning signs of pulmonary embolism. This is a medical emergency requiring an immediate call for emergency services. Exercise must stop right away and the client should be kept calm and still while help is on the way.

#355 · Client Safety and Client-Centred Care

Why avoid deep tissue massage directly over an area of suspected malignancy?

Answer

Deep pressure over a tumor site is generally avoided due to theoretical concerns about affecting local tissue and the client's overall condition, and because the underlying cause of the pain has not been medically ruled out. Any new unexplained mass or lump found during assessment should be referred for medical evaluation rather than treated directly. Treatment can often proceed elsewhere on the body once the client has appropriate medical clearance.

#356 · Client Safety and Client-Centred Care

Precaution for exercising a client on beta blockers

Answer

Beta blockers blunt the normal rise in heart rate during exercise, so heart rate alone is not a reliable measure of exertion in these clients. Perceived exertion scales and blood pressure response become more important for monitoring intensity safely. The therapist should know the client's medication list before setting exercise targets.

#357 · Client Safety and Client-Centred Care

Why check skin integrity before and after applying any modality?

Answer

Checking beforehand identifies existing wounds, rashes, or areas of poor sensation that change or rule out modality choice. Checking afterward catches early signs of a burn, blister, or adverse reaction before it worsens. This simple habit protects the client and gives the therapist objective documentation of the client's response.

#358 · Client Safety and Client-Centred Care

Metal implants and diathermy

Answer

Shortwave and microwave diathermy are contraindicated over or near metal implants because the metal can concentrate the electromagnetic energy and cause localized tissue burning. This includes joint replacements, surgical pins, and plates. Superficial heat or other modalities are safer alternatives in these areas.

#359 · Client Safety and Client-Centred Care

A pregnant client asks for a hot pack over her low back. What should the therapist do?

Answer

Direct heat over the abdomen and pelvis is avoided during pregnancy due to concerns about fetal effects from raised core temperature. Heat can still be applied cautiously to other areas of the body, and lower intensity or shorter duration is often used as an added precaution. The therapist should also confirm there is no other contraindication such as gestational hypertension before proceeding.

#360 · Client Safety and Client-Centred Care

Why is client education on home exercise safety part of injury prevention?

Answer

Clients who understand correct form, appropriate progression, and warning signs to stop are less likely to injure themselves between supervised sessions. Written or visual instructions reduce the chance of misremembering cues given verbally in the clinic. Reviewing the home program regularly catches unsafe technique before it becomes a habit.

#361 · Client Safety and Client-Centred Care

Universal design in a physiotherapy setting

Answer

Universal design means building spaces and equipment usable by the widest range of people from the start, such as height adjustable plinths, wide doorways, and clear signage, rather than adding accommodations later for specific clients. This benefits clients with disabilities, older adults, and people with temporary injuries alike. It reflects a proactive approach to accessibility rather than a reactive one.

#362 · Client Safety and Client-Centred Care

A client with a language barrier signs a consent form they may not fully understand. What is the concern?

Answer

A signature alone does not prove informed consent if the client could not understand the information in a language they follow well. This creates both an ethical problem and a legal vulnerability for the therapist and the clinic. Professional interpretation should be arranged before proceeding with any treatment that requires express consent.

#363 · Client Safety and Client-Centred Care

Monitoring a client during their very first exercise session after a recent cardiac event

Answer

Vital signs, symptoms, and perceived exertion should be checked at rest, during exercise, and during recovery, watching closely for chest pain, unusual shortness of breath, dizziness, or an abnormal blood pressure response. Intensity should start conservatively and progress gradually based on the client's response. Any concerning symptom means stopping the session and contacting the appropriate medical provider.

#364 · Client Safety and Client-Centred Care

Why document a client's refusal of a recommended treatment?

Answer

Documenting refusal shows the client was informed of the recommendation and the risks of declining it, and that their autonomous choice was respected. This protects both the client's right to decide and the therapist's professional record if the condition worsens later. The note should include what was explained and the client's stated reason if given.

#365 · Client Safety and Client-Centred Care

Core idea tying informed consent, red flag screening, and infection control together

Answer

All three exist to protect the client from preventable harm while respecting their right to make decisions about their own body and care. Consent respects autonomy, red flag screening catches serious hidden pathology, and infection control prevents avoidable transmission of disease. Together they form the safety foundation that client-centred physiotherapy practice is built on.

#366 · Collaboration

What is a physiotherapy assistant (PTA)?

Answer

A PTA is a support worker who carries out selected parts of a treatment plan under the direction and supervision of a physiotherapist. The physiotherapist stays responsible for assessment, diagnosis, planning, and the overall outcome. The PTA follows a plan the physiotherapist has already set, not one they create themselves.

#367 · Collaboration

Can a physiotherapy assistant perform an initial assessment?

Answer

No. Assessment, clinical reasoning, and diagnosis are core physiotherapist functions that cannot be delegated. The physiotherapist must complete the initial assessment and set the plan before any task goes to support personnel.

#368 · Collaboration

Scenario: a PTA reports a patient's pain got worse during a routine exercise session. What should the physiotherapist do?

Answer

The physiotherapist should re-assess the patient directly rather than asking the PTA to adjust the plan. A change in status means the case needs professional clinical judgment again. The physiotherapist then decides whether to modify, pause, or escalate the treatment.

#369 · Collaboration

Name three things that must happen before a physiotherapist delegates a task to support personnel.

Answer

The physiotherapist must confirm the patient is stable and the task is appropriate to delegate, confirm the support worker has the skill and training to do it safely, and give clear instructions plus a way to reach the physiotherapist if something changes. Without all three, delegation is not safe.

#370 · Collaboration

What does the term "competence" mean when assigning a task to a support worker?

Answer

Competence means the support worker has shown, through training or direct observation, that they can safely and correctly perform that specific task. It is task-specific, so being competent at one exercise does not mean being competent at all exercises. The physiotherapist should not assume competence, they should verify it.

#371 · Collaboration

True or false: once a physiotherapist delegates a task, they are no longer accountable for it.

Answer

False. The physiotherapist remains accountable for the outcome of any delegated task even though someone else performs it. Delegation transfers the doing of the task, not the professional responsibility for the patient's care.

#372 · Collaboration

What is the difference between direct and indirect supervision?

Answer

Direct supervision means the physiotherapist is on site and immediately available to step in. Indirect supervision means the physiotherapist is reachable by phone or other means but not physically present. The level needed depends on the patient's complexity and the support worker's experience.

#373 · Collaboration

Scenario: a new PTA has never worked with stroke patients before. The physiotherapist wants them to run a balance exercise with a stroke patient who has a fall risk. What level of supervision fits?

Answer

Direct supervision fits best here because the PTA lacks experience with this patient population and the task carries fall risk. The physiotherapist should stay present until the PTA demonstrates safe, consistent performance. Supervision level can be reduced once competence is confirmed.

#374 · Collaboration

Who typically leads discharge planning for a patient with complex home needs?

Answer

Discharge planning is a shared job, often coordinated with social work, nursing, and occupational therapy alongside the physiotherapist. Each discipline speaks to their own piece, such as mobility, equipment needs, home safety, or social supports. The team works together so the plan is realistic for the patient's actual living situation.

#375 · Collaboration

When should a physiotherapist refer a patient to occupational therapy?

Answer

Referral to OT fits when the patient's needs go beyond movement and strength, such as trouble with dressing, cooking, home modifications, or fine motor tasks. A physiotherapist should recognize the limits of their own scope and hand off pieces of care that fit another profession better. Waiting too long can slow the patient's overall recovery.

#376 · Collaboration

Scenario: a patient tells the physiotherapist they feel very low and have stopped eating. What is the appropriate next step?

Answer

The physiotherapist should ask more about the patient's mood and safety, then involve the physician, a mental health professional, or social work depending on urgency. Mental health and nutrition concerns fall outside physiotherapy scope and need proper referral. If there is any risk of self-harm, this becomes urgent and needs immediate escalation.

#377 · Collaboration

What is a scope of practice overlap?

Answer

A scope of practice overlap happens when more than one profession is legally allowed to perform a similar task, such as some wound care being done by both physiotherapists and nurses. When overlap exists, the team should agree on who does what to avoid gaps or repeated work. Clear communication prevents the patient from getting mixed messages.

#378 · Collaboration

Give an example of a task a physiotherapist can delegate to a PTA.

Answer

A physiotherapist can delegate routine, already-planned exercises like a set of strengthening reps, monitored ambulation using a pre-set assistive device, or applying a heat pack once the plan and precautions are established. These are tasks with a known, predictable response. Anything requiring new clinical judgment stays with the physiotherapist.

#379 · Collaboration

Give an example of a task a physiotherapist cannot delegate to a PTA.

Answer

A physiotherapist cannot delegate the initial assessment, diagnosis, setting or changing the treatment plan, or interpreting a change in the patient's condition. These all require professional clinical reasoning that only the physiotherapist is trained and licensed to provide. The PTA can report findings, but cannot decide what they mean.

#380 · Collaboration

What should feedback to a support worker sound like when a task was done incorrectly?

Answer

Feedback should be specific, timely, and focused on the behavior rather than the person, for example describing exactly what was missed and what the correct step looks like. It helps to check understanding by having the worker repeat back or demonstrate the correction. Vague comments like "do better" do not give the worker anything to act on.

#381 · Collaboration

Scenario: a support worker becomes defensive when given corrective feedback. What is a good approach?

Answer

The physiotherapist should stay calm, keep the conversation focused on patient safety and the specific task, and give the worker a chance to explain their reasoning. Framing feedback around a shared goal of good patient care reduces defensiveness. If the behavior repeats after clear feedback, it needs to be documented and escalated appropriately.

#382 · Collaboration

What is the sandwich method of feedback, and when is it not ideal?

Answer

The sandwich method places a criticism between two positive comments. It can soften a hard message but risks burying the real concern so the listener misses it, especially for safety issues. For anything involving patient safety, being direct and clear is more important than softening the message.

#383 · Collaboration

How often should a physiotherapist re-check a support worker's performance on a delegated task?

Answer

Re-checking should happen regularly, not just once, especially early in the working relationship or whenever the patient's condition changes. The frequency depends on task complexity and the worker's demonstrated consistency over time. Ongoing spot checks catch drift in technique before it becomes a safety problem.

#384 · Collaboration

What information must a physiotherapist give a support worker before assigning a task?

Answer

The physiotherapist must give clear instructions on what to do, how to do it, what signs mean stop and report back, and how to reach the physiotherapist if needed. This is sometimes called a clear directive. Without this, the support worker cannot safely carry out the task.

#385 · Collaboration

Scenario: a physician orders bed rest for a patient, but the physiotherapist believes early mobilization would help recovery. What should happen?

Answer

The physiotherapist should discuss their clinical reasoning directly with the physician rather than proceeding against the order or ignoring their own concern. Interprofessional conflict like this is resolved through respectful discussion of the evidence and the patient's specific situation. If they cannot agree, escalation through proper channels or a second opinion may be needed.

#386 · Collaboration

What is interprofessional collaboration?

Answer

Interprofessional collaboration is when different health professions work together, share information, and coordinate care so the patient gets consistent, safe treatment. It relies on clear communication, mutual respect, and understanding what each profession contributes. Poor collaboration leads to gaps, duplicated work, or conflicting instructions for the patient.

#387 • Collaboration

Why should a physiotherapist involve social work for a patient facing housing instability?

Answer

Housing instability affects a patient's ability to follow through with rehab, attend appointments, and recover safely, but it sits outside physiotherapy training and scope. Social work has the resources and skills to address these social needs. Referring early keeps the rehab plan realistic for the patient's actual circumstances.

#388 • Collaboration

Scenario: a nurse tells the physiotherapist a patient's blood pressure was unusually high just before a scheduled session. What should the physiotherapist do?

Answer

The physiotherapist should hold off on strenuous treatment and check the patient's current status before proceeding, since exercising with unstable blood pressure carries risk. Consulting with nursing or the physician about whether treatment is safe to continue is appropriate. Communication like this between disciplines is exactly what keeps patients safe.

#389 · Collaboration

What role does documentation play in interprofessional care?

Answer

Documentation lets other team members know what has been assessed, planned, and observed, so care stays consistent even when different providers see the patient at different times. Clear notes prevent repeated assessments and reduce the chance of conflicting instructions. Documentation is also part of the physiotherapist's professional accountability.

#390 · Collaboration

What should a physiotherapist do if a support worker performs a task outside their assigned scope?

Answer

The physiotherapist should stop the unsafe practice right away, address it directly with the worker, and document what happened. This protects the patient and clarifies expectations going forward. Repeated boundary violations may need to be reported through the workplace's proper channels.

#391 · Collaboration

Scenario: two team members disagree on whether a patient is ready to be discharged. How should this be resolved?

Answer

The disagreement should be worked out through a team discussion that looks at objective findings from each discipline rather than personal opinion. Each professional shares their specific concerns and evidence, and the group works toward a decision that reflects the patient's full picture. If needed, the case can be brought to a supervisor or team lead for a final call.

#392 · Collaboration

What is a key sign that a delegated task needs to come back to the physiotherapist?

Answer

Any unexpected change in the patient's pain, function, vital signs, or behavior during a delegated task is a sign it needs to go back to the physiotherapist. Support workers should be trained to recognize these red flags and stop the activity. The physiotherapist then reassesses before treatment continues.

#393 · Collaboration

Why is it important to know each team member's specific scope of practice?

Answer

Knowing each profession's scope avoids asking someone to do something outside their training, and avoids duplicating work another discipline is already covering. It also helps identify gaps where no one is addressing a patient need. Clear scope knowledge is the foundation of safe team based care.

#394 · Collaboration

Scenario: a physiotherapy student on placement wants to try a manual technique they just learned in school. What should the supervising physiotherapist consider?

Answer

The supervisor should assess whether the student has shown competence with that specific technique through practice and observation before allowing it on a patient. The supervisor stays present and ready to step in for a new technique. Supervision level should match the student's actual demonstrated skill, not their stage in school.

#395 • Collaboration

What does accountability mean for a physiotherapist supervising others?

Answer

Accountability means the physiotherapist answers for the quality and safety of the care given, even when a support worker or student physically performed the task. This is why proper training, clear instructions, and ongoing checks matter so much. The physiotherapist cannot hand off responsibility just by handing off the task.

#396 • Collaboration

Scenario: a family member asks the PTA questions about the patient's diagnosis and prognosis. What should the PTA do?

Answer

The PTA should redirect these questions to the physiotherapist since diagnosis and prognosis discussions are outside a support worker's role. The PTA can share general information about the exercises they are supervising, but not clinical interpretation. This keeps the family getting accurate information from the right source.

#397 · Collaboration

What is the purpose of a case conference or team huddle?

Answer

A case conference brings different disciplines together to share updates, align on goals, and catch problems before they grow. It gives everyone a chance to hear the full picture rather than just their own piece of the patient's care. Regular huddles improve coordination especially for complex or long-term patients.

#398 · Collaboration

Scenario: a support worker asks the physiotherapist to change a patient's exercise program because the patient complained it was too easy. What should the physiotherapist do?

Answer

The physiotherapist should reassess the patient rather than let the support worker adjust the program on their own. Changing a plan involves clinical judgment, which stays with the physiotherapist. Once reassessed, the physiotherapist can update the plan and communicate the new instructions clearly.

#399 · Collaboration

What is the difference between delegation and referral?

Answer

Delegation means assigning a specific task within the physiotherapist's own plan to a support worker who they still supervise. Referral means sending the patient to a different profession entirely, such as a physician or dietitian, for care outside physiotherapy scope. Delegation stays under the physiotherapist's responsibility, referral hands the concern to another provider.

#400 · Collaboration

Scenario: a physiotherapist notices a colleague from another discipline seems overwhelmed with a shared patient's care. What is a collaborative response?

Answer

The physiotherapist can offer to talk through the case, clarify what each of them is responsible for, and see if workload or communication issues are contributing. Supporting a colleague this way often prevents errors caused by burnout or miscommunication. Raising concerns respectfully protects both the patient and the working relationship.

#401 · Collaboration

Why should treatment goals be shared across the care team?

Answer

Shared goals keep every provider working in the same direction instead of giving the patient conflicting advice or duplicated effort. For example, if nursing and physiotherapy are both working on mobility, aligned goals prevent contradictory instructions about how much a patient should be doing. It also helps the patient understand and stay motivated toward one clear plan.

#402 · Collaboration

What should happen if a support worker is asked to perform a task they were never trained on?

Answer

The support worker should say so and decline the task rather than attempt it, and the physiotherapist should provide the training or perform the task themselves. Assigning untrained tasks puts the patient at risk and puts the support worker in an unfair position. Competence must be confirmed before, not after, the task is assigned.

#403 · Collaboration

Scenario: the physiotherapist and occupational therapist disagree about which one should address a patient's transfer training. What is a reasonable way to sort this out?

Answer

The two professionals should discuss their specific training and how each views the transfer issue, then agree on a plan that avoids duplicated or conflicting instruction to the patient. Whoever is best positioned for that piece of care, or both working together with a shared approach, should take it on. The patient benefits most when the team communicates rather than works in silos.

#404 · Collaboration

What is meant by "clear directive" in the context of delegation?

Answer

A clear directive is specific written or verbal instruction that tells the support worker exactly what to do, how to do it, and what to watch for. It removes guesswork and sets a standard the physiotherapist can check against later. Vague or general instructions increase the risk of errors.

#405 · Collaboration

Scenario: a patient asks the PTA to increase the intensity of their home exercise program because they feel ready. What should the PTA say?

Answer

The PTA should explain that changes to the program need to go through the physiotherapist and should pass the patient's request along. The PTA should not adjust exercise intensity on their own judgment. This keeps clinical decisions with the professional trained to make them.

#406 · Collaboration

Why should a physiotherapist document the rationale for delegating a specific task?

Answer

Documenting the rationale shows that the decision to delegate was based on the patient's stability and the worker's competence, not just convenience. It creates a record that supports safe practice if the case is ever reviewed. It also helps other team members understand what has already been decided about the patient's care.

#407 · Collaboration

What is a common cause of interprofessional conflict on a care team?

Answer

Unclear roles, poor communication, or differing opinions about the best plan for a patient are common causes of conflict between professions. Time pressure and lack of shared information can make these disagreements worse. Addressing conflict early with direct, respectful conversation usually prevents it from affecting patient care.

#408 · Collaboration

Scenario: a physiotherapist is asked to supervise a support worker who was hired and trained by a different department. What should the physiotherapist confirm first?

Answer

The physiotherapist should confirm the worker's actual competence for the specific tasks being assigned rather than assuming their prior training covers everything needed. Different departments may train to different standards. Direct observation is the most reliable way to confirm this before assigning independent work.

#409 · Collaboration

What is the physiotherapist's responsibility if a delegated task results in patient harm?

Answer

The physiotherapist remains professionally accountable for ensuring the delegation was appropriate, the worker was competent, and instructions were clear, even though someone else performed the task. This does not mean the support worker has no responsibility, but the physiotherapist's supervisory duty is always part of the review. This is why careful delegation decisions matter so much.

#410 · Collaboration

Scenario: a physiotherapist wants to assign wound dressing changes to a PTA. What should they check first?

Answer

The physiotherapist should check whether wound dressing changes fall within what a PTA is permitted to do in that setting and jurisdiction, and confirm the PTA has demonstrated the specific skill. Some wound care tasks may need to stay with nursing depending on complexity and local policy. When unsure, the safer choice is to keep the task with a more qualified provider or confirm with a supervisor.

#411 · Collaboration

What is the benefit of involving a physician early when a patient's pain pattern changes unexpectedly?

Answer

Early physician involvement helps rule out a medical cause that falls outside physiotherapy scope, such as an infection or a new pathology. Waiting too long can delay diagnosis and treatment of a serious underlying issue. Physiotherapists are trained to recognize warning signs and refer promptly rather than treat through them.

#412 · Collaboration

Scenario: a physiotherapist gives a support worker positive feedback for a task done well. Why does this matter for collaboration?

Answer

Positive feedback reinforces correct technique and builds trust, making the working relationship more effective over time. It also encourages the support worker to keep communicating openly about what they observe with patients. Feedback that only points out mistakes can discourage a worker from reporting concerns.

#413 · Collaboration

What should a physiotherapist do when they are unsure whether a task can be legally delegated in their province?

Answer

The physiotherapist should check their provincial regulatory college's standards or guidelines on delegation and support personnel before assigning the task. Rules can differ between provinces, so assuming based on general practice is risky. When still unsure, the safer choice is to perform the task themselves or ask the college directly.

#414 · Collaboration

Scenario: a rehab assistant on the team consistently skips checking in with the physiotherapist after sessions. What is the appropriate response?

Answer

The physiotherapist should address this directly with the assistant, restate the expectation for reporting back, and explain why it matters for patient safety. If the behavior continues, it should be documented and may need to be raised with a supervisor. Consistent check-ins are part of what makes delegation safe in the first place.

#415 · Collaboration

Why does effective interprofessional collaboration ultimately improve patient outcomes?

Answer

When professions share information and align on goals, patients get consistent instructions and fewer gaps or contradictions in their care. Problems are caught earlier because more trained eyes are watching for warning signs. Coordinated care also builds patient trust, since the team appears organized and on the same page.

#416 · Professional Responsibilities

What is the main job of a provincial physiotherapy regulatory college?

Answer

The college's job is to protect the public, not to represent physiotherapists as a group. It sets entry-to-practice standards, issues licenses, investigates complaints, and disciplines members who break the rules. This is different from a professional association, which lobbies and advocates for the profession's interests.

#417 · Professional Responsibilities

College vs professional association: what is the key difference?

Answer

A regulatory college exists to protect patients through licensing, standards, and discipline, and membership is mandatory to practice legally. A professional association is voluntary, and it focuses on advocacy, continuing education, and member benefits rather than public protection.

#418 · Professional Responsibilities

Name the four elements needed for informed consent to be valid.

Answer

The patient must be capable of understanding the decision, the consent must relate to the specific treatment offered, it must be given voluntarily without pressure, and it must be based on enough information about risks, benefits, and alternatives. If any one of these is missing, the consent is not valid.

#419 · Professional Responsibilities

A patient nods along during your explanation but you are not sure they understood. What should you do?

Answer

Ask the patient to repeat back the plan in their own words, a method called teach-back. Nodding is not proof of understanding, and a physiotherapist has a duty to confirm comprehension before proceeding, especially for higher-risk techniques like manipulation.

#420 · Professional Responsibilities

Can consent to physiotherapy treatment be implied rather than spoken out loud?

Answer

Yes, implied consent happens when a patient's actions show agreement, such as lying down on the table when asked. Implied consent is acceptable for routine, low-risk parts of treatment, but higher-risk procedures still need clear verbal or written consent.

#421 · Professional Responsibilities

What is PHIPA?

Answer

PHIPA stands for the Personal Health Information Protection Act, an Ontario law governing how health information custodians like physiotherapists collect, use, and share patient health information. Other provinces have similar privacy laws, but the underlying idea is the same across Canada: patient health information must be protected and only shared with proper authority.

#422 · Professional Responsibilities

A patient's spouse calls asking about the patient's treatment progress. What should you do?

Answer

Do not share any details unless the patient has given specific permission for that person to receive information. Confidentiality belongs to the patient, and even close family members do not automatically have a right to health details.

#423 · Professional Responsibilities

Name three situations where breaking patient confidentiality is legally justified.

Answer

A physiotherapist may or must disclose information when the patient consents, when a court order or subpoena requires it, or when there is a risk of serious harm to the patient or a third party such as suspected child abuse. These exceptions exist because public safety and legal duty can outweigh privacy in specific cases.

#424 · Professional Responsibilities

What is a conflict of interest in physiotherapy practice?

Answer

A conflict of interest happens when a physiotherapist's personal, financial, or other interests could improperly influence their professional judgment or actions toward a patient. Common examples include recommending unnecessary extra visits for revenue, or selling products to patients without disclosing a personal financial stake.

#425 · Professional Responsibilities

A physiotherapist owns a supplement company and recommends its products to every patient. What is the concern?

Answer

This is a conflict of interest because the therapist benefits financially from the recommendation, which may bias clinical judgment. At minimum the therapist must disclose the financial interest to the patient, and many colleges would still view this as inappropriate even with disclosure.

#426 · Professional Responsibilities

Boundary crossing vs boundary violation: what is the difference?

Answer

A boundary crossing is a minor, usually harmless departure from strict professional norms, like accepting a small homemade gift from a grateful patient. A boundary violation causes harm or exploits the power imbalance between therapist and patient, such as a romantic relationship with a current patient or accepting large sums of money.

#427 · Professional Responsibilities

Is a romantic relationship with a former patient automatically acceptable once treatment ends?

Answer

No. Most Canadian physiotherapy colleges require a defined cooling-off period after discharge, and even after that period the therapist must consider whether a power imbalance from the treatment relationship still exists. Entering a relationship too soon, or without addressing residual power imbalance, can still be considered professional misconduct.

#428 · Professional Responsibilities

Why does the power imbalance between therapist and patient matter for professional boundaries?

Answer

Patients are often physically vulnerable, in pain, or emotionally dependent during treatment, which gives the therapist inherent influence over them. Because of this imbalance, the responsibility to maintain appropriate boundaries always rests with the physiotherapist, not the patient.

#429 · Professional Responsibilities

What does evidence-based practice mean in physiotherapy?

Answer

Evidence-based practice means combining the best available research evidence with clinical expertise and the patient's own values and preferences. It is not just following research blindly, and it is not just doing what has always been done, it is a blend of all three sources of information.

#430 · Professional Responsibilities

Rank these from strongest to weakest evidence: expert opinion, randomized controlled trial, case report, systematic review.

Answer

From strongest to weakest: systematic review of randomized controlled trials, single randomized controlled trial, case report, then expert opinion. A systematic review pools many high-quality studies together, which usually gives the most reliable answer to a clinical question.

#431 · Professional Responsibilities

What is a systematic review, in plain terms?

Answer

A systematic review is a study that gathers all the relevant research on a specific question, using a clear and repeatable search method, then summarizes the combined findings. When it also statistically combines the numeric results, it is called a meta-analysis.

#432 · Professional Responsibilities

A physiotherapist keeps using a treatment technique just because it was taught in school 20 years ago, ignoring newer research showing it does not work. What principle is being violated?

Answer

This violates evidence-based practice, because clinical decisions should be updated as new, higher-quality research becomes available. Continuing to rely only on tradition or past training without reviewing current evidence can lead to outdated or ineffective care.

#433 · Professional Responsibilities

What does scope of practice mean for a physiotherapist?

Answer

Scope of practice is the set of activities a physiotherapist is legally and professionally allowed to perform, based on education, training, and provincial regulation. Acting outside this scope, even with good intentions, can be considered professional misconduct.

#434 · Professional Responsibilities

Can physiotherapists diagnose medical conditions?

Answer

Physiotherapists can perform a physiotherapy diagnosis, meaning they identify movement dysfunctions and determine if physiotherapy is appropriate. They cannot diagnose medical conditions such as diseases, and if a patient's presentation suggests a medical issue outside physiotherapy scope, referral to a physician is required.

#435 · Professional Responsibilities

A patient presents with red flag symptoms suggesting cauda equina syndrome. What is the physiotherapist's scope-of-practice duty here?

Answer

The physiotherapist must recognize the red flags are outside physiotherapy scope and refer the patient for emergency medical assessment right away. Treating the back pain as a routine musculoskeletal case instead of referring would be a serious scope-of-practice and patient safety failure.

#436 · Professional Responsibilities

What are restricted or controlled acts in the context of physiotherapy regulation?

Answer

Restricted acts are specific procedures, such as spinal manipulation or acupuncture with needles, that carry higher risk and can only be performed by professionals with the required authorization. A physiotherapist can only perform these acts if their license and additional certification specifically permit it.

#437 · Professional Responsibilities

What is self-reflection in the context of professional practice, and why does it matter?

Answer

Self-reflection is the ongoing process of examining one's own clinical decisions, biases, and gaps in knowledge to improve future practice. Regulatory colleges expect this because it drives continuing competence and helps a therapist recognize when they are practicing outside their comfort or skill level.

#438 · Professional Responsibilities

What should a physiotherapist do if they realize they lack the skill to treat a complex case?

Answer

The therapist should recognize the limitation through self-reflection and either seek mentorship, pursue additional training, or refer the patient to a colleague with the needed expertise. Continuing to treat beyond one's competence to avoid admitting a gap puts the patient at risk.

#439 · Professional Responsibilities

Define social determinants of health.

Answer

Social determinants of health are the non-medical factors that shape a person's health outcomes, such as income, housing, education, employment, and access to transportation. These factors often explain why two patients with the same diagnosis can have very different recovery paths.

#440 · Professional Responsibilities

A patient cannot attend twice-weekly physiotherapy sessions because they cannot afford transportation and work two jobs. How should this affect the plan?

Answer

The physiotherapist should adapt the plan around the patient's real-life constraints, such as offering a home exercise program, telehealth options, or a lower-frequency schedule. Ignoring social determinants of health and insisting on an ideal but unrealistic schedule usually leads to poor adherence and worse outcomes.

#441 · Professional Responsibilities

What is the difference between capacity and competence when discussing informed consent?

Answer

Capacity refers to a patient's ability to understand and appreciate the specific decision at hand, and it can change over time or vary by decision. Competence is a broader legal term about a person's general ability to manage their affairs, and in health care, capacity is the more relevant and commonly used concept.

#442 · Professional Responsibilities

Can a mature minor legally consent to physiotherapy treatment without a parent present?

Answer

In most Canadian provinces there is no fixed age of consent for health care, and a minor who demonstrates enough understanding of the treatment and its consequences can give valid consent themselves. This is often called the mature minor rule, though practice and documentation expectations vary by province.

#443 · Professional Responsibilities

What must happen before a physiotherapist can substitute a family member's consent for an incapable adult patient?

Answer

The therapist must first assess and document that the patient lacks capacity for the specific treatment decision. Once incapacity is established, consent shifts to a legally recognized substitute decision maker, following the priority order set out in that province's health care consent legislation.

#444 · Professional Responsibilities

What is the purpose of a physiotherapy code of ethics?

Answer

A code of ethics sets out the core values and principles, such as respect for patient autonomy, honesty, and doing no harm, that guide professional judgment when rules do not cover every situation. It works alongside standards of practice, which give more specific and enforceable behavioural expectations.

#445 · Professional Responsibilities

**Standards of practice vs code of ethics:
which one is more specific and
enforceable?**

Answer

Standards of practice are more specific and directly enforceable, describing exact expectations like documentation timelines or consent procedures. The code of ethics is broader and principle-based, offering guidance for judgment calls that standards do not explicitly cover.

#446 · Professional Responsibilities

**A physiotherapist discovers a colleague is
billing for sessions that never happened.
What is the professional obligation?**

Answer

Most Canadian colleges impose a duty to report suspected fraud or serious misconduct by another regulated health professional. Staying silent to avoid conflict does not satisfy this duty, and failing to report can itself be treated as a professional violation.

#447 · Professional Responsibilities

What is the purpose of clinical documentation from a regulatory standpoint?

Answer

Documentation creates a legal record of assessment, consent, treatment, and clinical reasoning that protects both the patient and the therapist. Regulatory colleges view poor or missing documentation as a standalone professional violation, separate from whether the actual care provided was appropriate.

#448 · Professional Responsibilities

A physiotherapist wants to try a new treatment technique they read about on social media that has no published research support. Is this appropriate?

Answer

This is risky because evidence-based practice requires weighing research evidence alongside clinical expertise and patient values, and a technique with no supporting evidence has not met that bar. The therapist should disclose the lack of evidence to the patient and get explicit informed consent if choosing to proceed at all.

#449 · Professional Responsibilities

What is the difference between an incidental finding and a red flag during a physiotherapy assessment?

Answer

An incidental finding is an unrelated observation that does not require urgent action, such as a mild skin blemish noticed during assessment. A red flag is a sign or symptom suggesting a serious underlying condition, such as unexplained weight loss or night pain, which requires prompt referral outside physiotherapy scope.

#450 · Professional Responsibilities

Why can accepting a large gift from a patient create a professional problem, even if the patient insists?

Answer

A large gift can create a conflict of interest and blur the professional boundary by shifting the relationship dynamic and creating a sense of obligation. Many colleges advise declining or redirecting gifts beyond a small token value, and documenting the situation if the patient is insistent.

#451 · Communication

What does building rapport with a new client actually involve in the first session?

Answer

Rapport starts with introducing yourself clearly, explaining your role, and asking the client what they hope to get out of treatment. It also means matching your tone and pace to how the client is responding, not just running through a scripted intake. A client who feels heard early on is far more likely to stay engaged with the plan of care.

#452 · Communication

Active listening

Answer

Active listening means giving full attention to what the client is saying, reflecting it back in your own words, and checking that you understood correctly before moving on. It includes noticing tone, pauses, and body language, not just the words. This confirms understanding and shows the client their concerns are actually being processed.

#453 · Communication

A client with early dementia keeps forgetting the home exercise instructions between visits. What should the physiotherapist adjust?

Answer

The physiotherapist should simplify instructions to one or two steps at a time, use pictures or large print handouts, and involve a caregiver or family member in the teaching. Repetition across several visits works better than a single detailed explanation. The goal is matching the format of the information to what the client can actually retain.

#454 · Communication

Why should clinical notes avoid vague terms like "doing better"?

Answer

Vague terms cannot be measured and give the next clinician nothing objective to compare against. Notes should state specific findings such as range of motion in degrees, pain rating on a scale, or distance walked. Objective language protects the client's record and supports any future review of the care given.

#455 · Communication

SOAP note

Answer

SOAP stands for Subjective, Objective, Assessment, and Plan. Subjective covers what the client reports, Objective covers measurable findings from the exam, Assessment covers the clinician's clinical reasoning, and Plan lays out next steps. This structure keeps documentation consistent so any reader can follow the clinical reasoning quickly.

#456 · Communication

A physiotherapist wants to text a client a reminder about tomorrow's appointment. What must happen first?

Answer

The client needs to give informed consent to being contacted by text, understanding that standard text messaging is not fully secure. The clinic should also confirm the phone number belongs to the client and not a shared family device. Sensitive clinical details should still be kept out of the text itself.

#457 · Communication

Telehealth communication barrier

Answer

Video visits remove the physiotherapist's ability to palpate or observe fine movement details in the same way an in-person visit allows. The clinician should ask the client to reposition the camera, describe sensations more precisely, and use verbal cueing more heavily to compensate. If the visual or verbal information is not enough to assess safely, an in-person visit should be arranged instead.

#458 · Communication

Why is it risky to correct a client's grammar or restate everything in medical jargon during an assessment?

Answer

Jargon-heavy language can make the client feel talked down to or confused, which shuts down honest reporting of symptoms. The physiotherapist should mirror the client's own words when documenting and asking follow up questions. Plain language keeps the client an active partner in the assessment rather than a passive listener.

#459 · Communication

A client speaks limited English and no interpreter is booked for today's session. What is the appropriate next step?

Answer

The physiotherapist should reschedule non-urgent care until a qualified interpreter can be arranged rather than relying on a family member for medical translation. Family interpretation risks inaccurate translation and can compromise the client's privacy and comfort discussing symptoms. If the situation is urgent, a phone interpretation service should be used immediately.

#460 · Communication

Closed loop communication

Answer

Closed loop communication means the sender gives an instruction, the receiver repeats it back to confirm, and the sender verifies the repeat back was correct. This is especially useful when handing off a client between team members or giving instructions to a support worker. It reduces the chance that a detail gets lost or misheard along the way.

#461 · Communication

Why should late entries in a chart be labeled as late entries rather than backdated?

Answer

Backdating a note misrepresents when the information was actually recorded, which can raise serious credibility concerns if the record is ever reviewed. A late entry should be added with the current date and time, clearly marked as a late entry, and reference the date of the original visit. This keeps the timeline of the record honest and defensible.

#462 · Communication

A frustrated client raises their voice about a delayed appointment. What is the best initial response?

Answer

The physiotherapist should acknowledge the frustration directly, avoid becoming defensive, and let the client finish speaking before responding. A calm tone and a clear explanation of what happened, followed by what will be done differently, usually de-escalates the situation. Arguing back or dismissing the complaint tends to make the client feel unheard and escalates conflict.

#463 · Communication

Nonverbal communication in physiotherapy

Answer

Nonverbal cues include facial expression, posture, eye contact, and guarding during movement testing, all of which can reveal pain or fear that a client does not say out loud. A physiotherapist should watch for these cues throughout the session, not just rely on the client's verbal pain rating. Mismatch between what is said and what the body shows is often clinically meaningful.

#464 · Communication

Why should email communication with clients include a confidentiality notice and be sent through a secure platform when possible?

Answer

Standard email is not guaranteed to be secure in transit and can be intercepted or sent to the wrong address by mistake. A confidentiality notice and a secure encrypted platform reduce the risk of a privacy breach if that happens. Sensitive clinical information should be minimized in email regardless, with detailed discussion saved for a call or visit.

#465 · Communication

A client with a hearing impairment is booked for assessment. What communication adjustments help most?

Answer

Facing the client directly so they can read lips, speaking at a normal pace without shouting, and reducing background noise all help. Written instructions or diagrams as backup are useful since some spoken words may be missed. Asking the client which method works best for them is more reliable than guessing.

#466 · Communication

Motivational interviewing

Answer

Motivational interviewing is a communication style that helps a client explore their own reasons for change rather than being told what to do. It uses open questions, reflective listening, and affirmations to build the client's own motivation for exercise or lifestyle adherence. It works well for clients who are ambivalent about committing to a home program.

#467 • Communication

Why should a physiotherapist avoid giving a firm diagnosis or prognosis over a quick hallway conversation with a worried family member?

Answer

Hallway conversations are not documented and often happen without the client's presence or explicit consent to share details. Clinical information should be discussed in a proper setting with the client's permission, and any conversation that does happen should still be documented afterward. This protects both the client's privacy and the accuracy of what was actually communicated.

#468 • Communication

A client discloses something in confidence during treatment that suggests risk of harm to themselves. What communication duty applies?

Answer

The physiotherapist has a duty to break confidentiality when there is a genuine risk of harm and must escalate to appropriate supports such as a crisis line, physician, or emergency service. The client should be told, when safely possible, that this information cannot stay confidential given the risk involved. The disclosure and the actions taken must be documented clearly and promptly.

#469 · Communication

Health literacy

Answer

Health literacy is a client's ability to understand and use health information to make decisions about their own care. A physiotherapist should check understanding using teach back, where the client explains the instructions back in their own words. Assuming understanding without checking is a common cause of poor home program adherence.

#470 · Communication

Why is documenting exactly what a client said, in quotation marks when relevant, sometimes preferred over paraphrasing?

Answer

Direct quotes preserve the client's own description of pain or function without the clinician's interpretation altering the meaning. This is especially useful for symptoms that carry legal or insurance significance, such as after a motor vehicle accident. Paraphrasing still has a place for general summaries, but exact wording protects accuracy where it matters most.

#471 · Communication

A client asks the physiotherapist to email their assessment report directly to a lawyer. What should happen before sending anything?

Answer

The physiotherapist needs a signed release of information from the client authorizing that specific disclosure before sending any report. The email should go through a secure channel and only include what was authorized, not the full chart. Without written consent, sending clinical information to a third party is a privacy breach.

#472 · Communication

Adjusting communication style for a pediatric client

Answer

Communication with children should use simple concrete language, short sentences, and often play or demonstration instead of verbal explanation alone. Involving the parent or guardian as a translator of instructions between visits reinforces the plan. Tone and body language matter as much as word choice since young children often respond to how something is said.

#473 · Communication

Why should a discharge summary be written in language the referring physician and the client can both understand?

Answer

A discharge summary serves two audiences at once, the referring provider who needs clinical detail and the client who needs to know their status in plain terms. Overly technical language can leave the client confused about their own recovery, while overly simplified language can leave the referring provider without enough clinical detail. A well written summary balances both without sacrificing accuracy.

#474 · Communication

A client cancels three appointments in a row by text with no explanation. What communication step is appropriate before discharging them from the caseload?

Answer

The physiotherapist should attempt a direct conversation, by phone or in person if possible, to understand the reason for the cancellations before assuming disengagement. Barriers like transportation, cost, or worsening symptoms are common and may be addressable. Only after a genuine attempt to reconnect should discharge for non-attendance be documented and communicated clearly to the client.

#475 · Communication

Using an interpreter service during a physiotherapy assessment

Answer

The physiotherapist should speak directly to the client, not to the interpreter, using short clear sentences that are easy to translate accurately. The interpreter should be a qualified professional rather than a friend or family member whenever the situation allows it. Pausing regularly gives the interpreter time to translate fully instead of summarizing content.

#476 · Practice Management

How long must a physiotherapy clinical record generally be retained for an adult client in most Canadian jurisdictions?

Answer

Most Canadian physiotherapy regulatory colleges require records to be kept for at least ten years from the date of the last entry. Specific retention periods can vary slightly by province, so the physiotherapist should confirm the exact rule with their own regulatory college. The record must remain accessible and legible for the full retention period, not just stored somewhere.

#477 · Practice Management

Records retention for a pediatric client

Answer

Records for clients who were minors at the time of treatment are typically retained for a longer period, often ten years past the age of majority rather than ten years from the last visit. This accounts for the possibility that a claim or question about the treatment could arise well into the client's adulthood. Clinics should track pediatric records separately so retention dates are not miscalculated.

#478 · Practice Management

A clinic is closing permanently. What must happen to the existing client records?

Answer

The clinic owner must arrange secure storage or transfer of all records for the remainder of the required retention period, and notify current clients how they can access their records going forward. Simply discarding records when a clinic closes is not acceptable under regulatory requirements. Many colleges require notification of the closure plan in advance.

#479 · Practice Management

Why should outcome measure scores be recorded at both initial assessment and regular intervals, not just at discharge?

Answer

Repeated measurement over time is what actually shows whether the client is improving, plateauing, or declining. A single score at discharge cannot demonstrate progress or justify why treatment continued as long as it did. Tracked scores also give objective evidence if a payer or insurer questions the necessity of continued care.

#480 · Practice Management

A client's functional outcome score has not changed in the last four sessions. What should the physiotherapist consider?

Answer

A plateau in outcome scores is a signal to reassess the plan of care rather than continuing the same approach indefinitely. The physiotherapist should determine whether the goals or techniques need to change, or whether the client has reached maximal benefit and is ready for discharge. Continuing unchanged treatment despite no measurable progress is difficult to justify if reviewed.

#481 · Practice Management

Workplace hazard identification

Answer

Hazard identification means actively looking for anything in the clinic environment that could cause injury, such as frayed electrical cords, wet floors, cluttered walkways, or poorly maintained equipment. It should be done on a regular schedule, not only after an incident occurs. Identified hazards need a documented plan for correction and a timeline for follow up.

#482 · Practice Management

A physiotherapist notices a treatment table with a loose bolt that wobbles under load. What is the correct next step?

Answer

The table should be taken out of service immediately and tagged so no one else uses it until it is repaired. The hazard should be reported to whoever manages equipment maintenance and documented in the clinic's hazard log. Continuing to use equipment with a known safety defect exposes both the client and the clinic to unnecessary risk.

#483 · Practice Management

Why is caseload different from workload when a physiotherapist is deciding how many clients to accept?

Answer

Caseload refers to the total number of clients assigned, while workload also accounts for the complexity, acuity, and time demands each client actually requires. A physiotherapist with a smaller caseload of highly complex clients can be just as stretched as one with a larger caseload of simple cases. Decisions about accepting new clients should weigh workload capacity, not just the raw caseload number.

#484 · Practice Management

A physiotherapist's caseload is full but a new referral describes a client with a rapidly worsening neurological condition. What should happen?

Answer

Urgency and clinical need should influence prioritization, so the new referral may need to be triaged ahead of less urgent existing clients on the waitlist. The physiotherapist should review current caseload for anyone ready for discharge or transfer to make room safely. Client safety takes priority over strict first come first served waitlist order when there is a clear urgent need.

#485 · Practice Management

Waitlist management

Answer

A waitlist should be reviewed regularly, not left static, since client needs and urgency can change over time. Clients waiting longer than expected should be contacted periodically to confirm they still need the service and check whether their condition has changed. A waitlist that is never reviewed risks either over promising availability or under serving clients who have become more urgent.

#486 · Practice Management

Why should a clinic track outcome data across its whole client population, not just individual client files?

Answer

Aggregated outcome data lets the clinic see patterns, such as which conditions respond well to certain approaches and which do not, informing quality improvement. It also supports accountability to funders or insurers who may ask for evidence the clinic's services are effective overall. Individual files alone cannot show trends across the practice.

#487 · Practice Management

A client insists on continuing weekly visits even though their outcome score has reached the normal range for their age group. What is the appropriate business practice?

Answer

The physiotherapist should discuss the outcome data directly with the client and explain that continued visits are no longer clinically justified. Discharge with a home maintenance program is the appropriate plan once functional goals are met. Continuing to bill for visits without clinical justification raises both ethical and business practice concerns.

#488 · Practice Management

Business practice standard for informed financial consent

Answer

Clients should be told the cost of services, what insurance covers, and any expected out of pocket cost before treatment begins, not after the fact. This should be documented, ideally with the client's signature or written acknowledgment. Surprise billing damages trust and can trigger a complaint to the regulatory college.

#489 · Practice Management

Why should incident reports from workplace hazards be reviewed at a clinic level, not just filed away?

Answer

Reviewing incident reports as a group reveals whether the same type of hazard keeps recurring, pointing to a systemic problem rather than a one time accident. This supports proactive changes like equipment replacement or workflow redesign before someone gets hurt. Filing reports without review turns a valuable safety tool into paperwork with no benefit.

#490 · Practice Management

A staff physiotherapist repeatedly lifts heavy equipment alone despite a policy requiring two person lifts. What hazard management step applies?

Answer

This is a hazard from unsafe work practice, not just equipment, and needs to be addressed through direct feedback and retraining on the two person lift policy. Management should confirm the policy is realistic given staffing levels, since an unworkable policy invites shortcuts. The issue should be documented and monitored until the safe practice is consistently followed.

#491 · Practice Management

Records retention when a client transfers ownership of their care between two clinics

Answer

The original clinic still must retain its own copy of the record for the full required retention period even after a transfer copy is sent to the new provider. A transfer of care is not the same as a transfer of the legal recordkeeping obligation. The originating clinic remains responsible for its documentation regardless of where the client goes next.

#492 · Practice Management

Why should outcome measures be standardized and validated tools rather than a clinician's own informal rating scale?

Answer

Standardized validated tools have known reliability and can be compared across clients, clinicians, and time in a meaningful way. An informal rating scale is subjective and cannot be defended the same way if care is reviewed by an insurer or regulatory body. Using recognized tools also allows outcomes to be benchmarked against published normative data.

#493 · Practice Management

A physiotherapist is asked to see twelve new clients this week on top of an already full schedule. What should guide the decision to accept or decline?

Answer

The physiotherapist should weigh whether accepting the new clients compromises the quality or safety of care for existing clients already on the caseload. Declining or delaying new intakes is appropriate when capacity is genuinely exceeded, and this should be communicated honestly rather than rushing everyone through shorter visits. Overextending caseload to avoid saying no can lead to missed hazards and poor outcomes across the board.

#494 · Practice Management

Slip and fall hazard in a hydrotherapy or pool based clinic area

Answer

Wet floor surfaces around a pool require slip resistant flooring, clear signage, and a routine drying or mopping schedule to reduce fall risk. Staff and clients moving between wet and dry areas are at the highest risk and should be reminded to move slowly. Regular hazard walkthroughs of this area should specifically check flooring condition and signage visibility.

#495 · Practice Management

Why does using outcome data to justify discharge protect the physiotherapist as well as the client?

Answer

Objective data showing the client has met functional goals gives a clear, defensible reason for ending treatment if the decision is ever questioned by the client, a payer, or a regulatory body. It shifts the conversation away from subjective opinion toward measurable evidence. This also helps the client understand the decision is based on their own progress, not an arbitrary cutoff.

#496 · Practice Management

A new graduate physiotherapist wants to know how to prioritize a growing waitlist fairly. What is a reasonable business practice?

Answer

A documented triage system based on clinical urgency, functional risk, and time already waited gives a consistent and defensible way to prioritize. Relying purely on the order referrals arrived ignores clinical need and can leave urgent cases waiting behind stable ones. The triage criteria should be written down and applied consistently by everyone in the clinic.

#497 · Practice Management

Records retention for records involved in an active complaint or legal claim

Answer

Records connected to an ongoing complaint, lawsuit, or investigation must be retained beyond the normal retention period until the matter is fully resolved. Destroying or altering a record during an active claim can itself lead to serious professional consequences. The clinic should flag any file under review so it is excluded from routine destruction schedules.

#498 · Practice Management

Why should a clinic keep a written log of workplace hazard reports and corrective actions rather than handling each one verbally?

Answer

A written log creates a paper trail showing hazards were identified and addressed, which matters if a workplace safety inspection or injury claim occurs later. Verbal only handling relies on memory and can leave no proof the clinic acted responsibly. The log also helps track whether the same hazard keeps happening despite previous fixes.

#499 · Practice Management

A physiotherapist owns a small private clinic and wants to set fees. What business practice consideration applies?

Answer

Fees should be set transparently, published clearly for clients to see, and applied consistently rather than negotiated case by case in a way that seems arbitrary. Any fee schedule should also account for what is reasonable and competitive for the local market and service provided. Clients should never be surprised by a fee that was not disclosed before their appointment.

#500 · Practice Management

Using outcome data trends to decide whether a specific treatment technique should remain part of a clinic's standard practice

Answer

If outcome data across many clients shows a technique consistently produces poor or no improvement, that is a signal the clinic should reconsider offering it as standard practice. This kind of decision should be based on aggregated evidence, not the impression from one or two memorable cases. Regularly reviewing technique level outcomes keeps the clinic's practice aligned with what actually works for its clients.